



**BETH ISRAEL LAHEY HEALTH, INC.
AND AFFILIATES**

Independent Auditors' Reports as Required by Title 2
U.S. Code of Federal Regulations Part 200, *Uniform Administrative
Requirements, Cost Principles, and Audit Requirements for Federal
Awards and Government Auditing Standards and Related Information*

Year Ended September 30, 2024

**BETH ISRAEL LAHEY HEALTH, INC.
AND AFFILIATES**

Independent Auditors' Reports as Required by Title 2
U.S. Code of Federal Regulations Part 200, Uniform Administrative Requirements, Cost Principles, and Audit
Requirements for Federal Awards and *Government Auditing Standards* and Related Information
Year ended September 30, 2024

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KPMG LLP
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Boston, MA 02111

Independent Auditors' Report

Board of Trustees
Beth Israel Lahey Health, Inc. and Affiliates:

Report on the Audit of the Consolidated Financial Statements

Opinion

We have audited the consolidated financial statements of Beth Israel Lahey Health, Inc. and Affiliates (BILH), which comprise the consolidated balance sheets as of September 30, 2024 and 2023, and the related consolidated statements of operations, changes in net assets, and cash flows for the years then ended, and the related notes to the consolidated financial statements.

In our opinion, the accompanying consolidated financial statements present fairly, in all material respects, the financial position of BILH as of September 30, 2024 and 2023, the results of its operations, changes in its net assets, and its cash flows for the years then ended in accordance with U.S. generally accepted accounting principles.

Basis for Opinion

We conducted our audits in accordance with auditing standards generally accepted in the United States of America (GAAS) and the standards applicable to financial audits contained in *Government Auditing Standards*, issued by the Comptroller General of the United States. Our responsibilities under those standards are further described in the Auditors' Responsibilities for the Audit of the Consolidated Financial Statements section of our report. We are required to be independent of BILH and to meet our other ethical responsibilities, in accordance with the relevant ethical requirements relating to our audits. We believe that the audit evidence we have obtained is sufficient and appropriate to provide a basis for our audit opinion.

Responsibilities of Management for the Consolidated Financial Statements

Management is responsible for the preparation and fair presentation of the consolidated financial statements in accordance with U.S. generally accepted accounting principles, and for the design, implementation, and maintenance of internal control relevant to the preparation and fair presentation of consolidated financial statements that are free from material misstatement, whether due to fraud or error.

In preparing the consolidated financial statements, management is required to evaluate whether there are conditions or events, considered in the aggregate, that raise substantial doubt about BILH's ability to continue as a going concern for one year after the date the consolidated financial statements are issued.

Auditors' Responsibilities for the Audit of the Consolidated Financial Statements

Our objectives are to obtain reasonable assurance about whether the consolidated financial statements as a whole are free from material misstatement, whether due to fraud or error, and to issue an auditors' report that includes our opinion. Reasonable assurance is a high level of assurance but is not absolute assurance and therefore is not a guarantee that an audit conducted in accordance with GAAS and *Government Auditing Standards* will always detect a material misstatement when it exists. The risk of not detecting a material misstatement resulting from fraud is higher than for one resulting from error, as fraud may involve collusion, forgery, intentional omissions, misrepresentations, or the override of internal control. Misstatements are



considered material if there is a substantial likelihood that, individually or in the aggregate, they would influence the judgment made by a reasonable user based on the consolidated financial statements.

In performing an audit in accordance with GAAS and *Government Auditing Standards*, we:

- Exercise professional judgment and maintain professional skepticism throughout the audit.
- Identify and assess the risks of material misstatement of the consolidated financial statements, whether due to fraud or error, and design and perform audit procedures responsive to those risks. Such procedures include examining, on a test basis, evidence regarding the amounts and disclosures in the consolidated financial statements.
- Obtain an understanding of internal control relevant to the audit in order to design audit procedures that are appropriate in the circumstances, but not for the purpose of expressing an opinion on the effectiveness of BILH's internal control. Accordingly, no such opinion is expressed.
- Evaluate the appropriateness of accounting policies used and the reasonableness of significant accounting estimates made by management, as well as evaluate the overall presentation of the consolidated financial statements.
- Conclude whether, in our judgment, there are conditions or events, considered in the aggregate, that raise substantial doubt about BILH's ability to continue as a going concern for a reasonable period of time.

We are required to communicate with those charged with governance regarding, among other matters, the planned scope and timing of the audit, significant audit findings, and certain internal control related matters that we identified during the audit.

Other Reporting Required by *Government Auditing Standards*

In accordance with *Government Auditing Standards*, we have also issued our report dated December 17, 2024 on our consideration of BILH's internal control over financial reporting and on our tests of its compliance with certain provisions of laws, regulations, contracts, and grant agreements and other matters. The purpose of that report is solely to describe the scope of our testing of internal control over financial reporting and compliance and the results of that testing, and not to provide an opinion on the effectiveness of BILH's internal control over financial reporting or on compliance. That report is an integral part of an audit performed in accordance with *Government Auditing Standards* in considering BILH's internal control over financial reporting and compliance.

KPMG LLP

Boston, Massachusetts
December 17, 2024

**BETH ISRAEL LAHEY HEALTH, INC.
AND AFFILIATES**

Consolidated Balance Sheets

September 30, 2024 and 2023
(In thousands)

Assets	2024	2023
Current assets:		
Cash and cash equivalents	\$ 328,294	175,622
Investments	2,225,877	2,127,730
Patient accounts receivable	924,776	722,681
Other current assets	450,797	425,659
Total current assets	3,929,744	3,451,692
Assets limited or restricted as to use:		
Held by trustees under debt and other agreements	194,258	62,804
Held for specific purposes and endowments	1,015,584	818,421
Total assets limited or restricted as to use	1,209,842	881,225
Long-term investments	107,358	92,408
Property and equipment, net	2,867,576	2,712,531
Professional liability reinsurance recoveries	208,599	202,001
Right-of-use operating lease assets, net	501,094	515,200
Other assets	195,595	161,359
Total assets	\$ 9,019,808	8,016,416
Liabilities and Net Assets		
Current liabilities:		
Current portion of long-term debt	\$ 40,718	55,996
Short-term borrowings	118,300	—
Current portion of operating lease obligations	97,653	86,832
Accounts payable and accrued expenses	1,263,423	1,058,422
Estimated settlements with third-party payors	170,424	170,431
Total current liabilities	1,690,518	1,371,681
Long-term debt, net of current portion	1,983,731	1,612,180
Operating lease obligations, net of current portion	424,571	447,942
Professional liability	304,382	287,783
Employee benefit plan liabilities	203,122	236,866
Other liabilities	169,360	91,322
Total liabilities	4,775,684	4,047,774
Net assets:		
Without donor restrictions	3,272,270	3,150,221
With donor restrictions	971,854	818,421
Total net assets	4,244,124	3,968,642
Total liabilities and net assets	\$ 9,019,808	8,016,416

See accompanying notes to consolidated financial statements.

**BETH ISRAEL LAHEY HEALTH, INC.
AND AFFILIATES**

Consolidated Statements of Operations
Years ended September 30, 2024 and 2023
(In thousands)

	2024	2023
Operating revenues:		
Patient service revenue	\$ 7,101,399	6,439,324
Research revenue	319,683	283,256
Other revenue	1,486,102	971,578
Total operating revenues	<u>8,907,184</u>	<u>7,694,158</u>
Operating expenses:		
Salaries and benefits	5,217,582	4,696,382
Supplies and other expenses	3,554,449	2,797,579
Depreciation	310,236	282,646
Interest	73,868	48,767
Total operating expenses	<u>9,156,135</u>	<u>7,825,374</u>
Loss from operations	<u>(248,951)</u>	<u>(131,216)</u>
Nonoperating gains (losses):		
Investment income, net	360,140	189,132
Pension costs	(39,152)	(46,890)
Effects of acquisition	273	242,880
Other	4,332	2,086
Nonoperating gains (losses), net	<u>325,593</u>	<u>387,208</u>
Excess of revenues over expenses	<u>76,642</u>	<u>255,992</u>
Net assets released from restrictions used for purchase of property and equipment	12,834	99,870
Change in funded status of employee benefit plans, other than net periodic benefit cost	<u>32,573</u>	<u>95,346</u>
Increase in net assets without donor restrictions	<u>\$ 122,049</u>	<u>451,208</u>

See accompanying notes to consolidated financial statements.

**BETH ISRAEL LAHEY HEALTH, INC.
AND AFFILIATES**

Consolidated Statements of Changes in Net Assets

Years ended September 30, 2024 and 2023
(In thousands)

	Without donor restrictions	With donor restrictions	Total
Net assets as of September 30, 2022	\$ 2,699,013	795,399	3,494,412
Excess of revenues over expenses	255,992	—	255,992
Restricted contributions, net	—	62,884	62,884
Restricted investment gains, net	—	48,815	48,815
Net assets released from restrictions used for operations	—	(13,866)	(13,866)
Net assets released from restrictions used for purchase of property and equipment	99,870	(99,870)	—
Change in funded status of employee benefit plans, other than net periodic benefit cost	95,346	—	95,346
Effects of acquisition – Exeter Health Resources	—	25,059	25,059
Increase in net assets	<u>451,208</u>	<u>23,022</u>	<u>474,230</u>
Net assets as of September 30, 2023	3,150,221	818,421	3,968,642
Excess of revenues over expenses	76,642	—	76,642
Restricted contributions, net	—	132,958	132,958
Restricted investment gains, net	—	52,956	52,956
Net assets released from restrictions used for operations	—	(19,647)	(19,647)
Net assets released from restrictions used for purchase of property and equipment	12,834	(12,834)	—
Change in funded status of employee benefit plans, other than net periodic benefit cost	32,573	—	32,573
Increase in net assets	<u>122,049</u>	<u>153,433</u>	<u>275,482</u>
Net assets as of September 30, 2024	<u>\$ 3,272,270</u>	<u>971,854</u>	<u>4,244,124</u>

See accompanying notes to consolidated financial statements.

**BETH ISRAEL LAHEY HEALTH, INC.
AND AFFILIATES**

Consolidated Statements of Cash Flows
Years ended September 30, 2024 and 2023
(In thousands)

	2024	2023
Cash flows from operating activities:		
Change in net assets	\$ 275,482	474,230
Adjustments to reconcile change in net assets to net cash used in operating activities:		
Effects of acquisition	—	(267,939)
Loss on extinguishment of debt	919	—
Change in funded status of employee benefit plans	(32,573)	(95,346)
Depreciation	310,236	282,646
Amortization	(7,609)	(10,075)
Loss on disposal of assets	1,955	—
Net gains on investments	(413,096)	(237,947)
Restricted contributions	(16,096)	(62,884)
(Decrease) increase in cash, net of effect of acquisition, resulting from changes in:		
Patient accounts receivable	(202,095)	(108,477)
Other current assets	(25,573)	(77,902)
Accounts payable and accrued expenses	74,037	36,692
Estimated settlements with third-party payors	(7)	13,112
Contributions receivable	(17,162)	(4,365)
Other assets and liabilities	55,041	49,760
Net cash provided by (used in) operating activities	<u>3,459</u>	<u>(8,495)</u>
Cash flows from investing activities:		
Purchases of property and equipment	(335,601)	(390,947)
Exeter cash at time of acquisition	—	17,882
Sales of investments and assets whose use is limited or restricted	1,087,856	862,976
Purchases of investments and assets whose use is limited or restricted	<u>(1,095,283)</u>	<u>(524,982)</u>
Net cash used in investing activities	<u>(343,028)</u>	<u>(35,071)</u>
Cash flows from financing activities:		
Borrowings under taxable commercial paper	420,275	—
Repayments of borrowings under taxable commercial paper	(305,900)	—
Proceeds from long-term debt	442,970	—
Payments on long-term debt	(71,915)	(43,376)
Debt issuance costs	(3,559)	—
Restricted contributions	<u>16,096</u>	<u>62,884</u>
Net cash provided by financing activities	<u>497,967</u>	<u>19,508</u>
Net increase (decrease) in cash and cash equivalents	<u>158,398</u>	<u>(24,058)</u>
Cash and cash equivalents at beginning of year	<u>264,723</u>	<u>288,781</u>
Cash and cash equivalents at end of year	<u>\$ 423,121</u>	<u>264,723</u>
The consolidated cash and cash equivalents at the end of the year include the following:		
Cash and cash equivalents within the consolidated balance sheet	\$ 328,294	175,622
Restricted cash included in assets held for specific purposes and endowments within the consolidated balance sheet	<u>94,827</u>	<u>89,101</u>
Cash, cash equivalents, and restricted cash at the end of the year	<u>\$ 423,121</u>	<u>264,723</u>
Supplemental disclosures of cash flow information:		
Change in noncash acquisitions of property and equipment	\$ 130,964	2,223

See accompanying notes to consolidated financial statements.

**BETH ISRAEL LAHEY HEALTH, INC.
AND AFFILIATES**

Notes to Consolidated Financial Statements

September 30, 2024 and 2023

(In thousands)

(1) Organization and Mission

The Beth Israel Lahey Health system was formed on March 1, 2019 through the combination of the hospitals and other affiliates of three legacy health care systems based primarily in the Eastern Massachusetts market, including the former CareGroup health system, the former Lahey health system, and the Seacoast health system. Under an affiliation agreement effective November 1, 2021, BILH became the sole corporate member of Joslin Diabetes Center, Inc. ("Joslin") and its affiliates. Additionally, under an affiliation agreement effective July 1, 2023, BILH became the sole corporate member of Exeter Health Resources, Inc. ("EHR" or "Exeter") and its affiliates (see note 17). These combined legacy health systems and their affiliates are collectively referred to herein as "BILH" or the "System."

BILH was formed by the creation of a new common parent corporation, Beth Israel Lahey Health, Inc. (the "Sole Member"), which became the direct or indirect controlling entity of the corporations constituting the System. The Sole Member's corporate purpose is to further the common and unifying charitable health care mission of the legacy organizations to promote and improve the quality, and expand the scope and accessibility, of affordable health care and health-care related services for the communities they serve.

BILH is the second largest health care system in New England and includes an academic medical center, two additional general acute care teaching hospitals, an orthopedic teaching hospital, nine acute care community hospitals, a behavioral health hospital, and a full continuum of services including primary and specialty care, community acute care, ambulatory care, behavioral health, and home health spanning a broad geography from Cape Cod to Southern New Hampshire.

BILH's vision is to serve as an integrated delivery system for its patients in order to:

- Improve the health and wellness of the populations BILH serves through integrated, patient-centered care;
- Create a transformative model of care, providing the highest quality care in appropriate settings at a competitive price;
- Optimize the use of both community and academic/tertiary institutions and providers;
- Support and value both independent and employed physicians in a common clinical enterprise;
- Maintain an attractive and supportive environment for clinicians, professional staff and employees; and
- Advance the science and practice of medicine through path-breaking research and education.

Northeast Behavioral Health ("NBH") is a not-for-profit corporation that provides outpatient services, community support services, youth residential services, substance abuse services, trauma services, and medical services in the northeast area of Massachusetts. NBH is the sole member of CAB Health and Recovery Services, Inc., which is a not-profit corporation organized for the prevention of substance abuse and the rehabilitation of addicted individuals throughout the greater north shore area in Massachusetts, and of Health and Education Housing Services, Inc.

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Notes to Consolidated Financial Statements

September 30, 2024 and 2023

(In thousands)

On September 14, 2023, Dana-Farber Cancer Institute, Inc. ("DFCI") announced a proposed relocation and expansion of its current adult inpatient hospital facility to a new, freestanding facility dedicated exclusively to the provision of adult inpatient cancer treatment and a clinical collaboration with BILH, Beth Israel Deaconess Medical Center, Inc. ("BIDMC"), and Harvard Medical Faculty Physicians at Beth Israel Deaconess Medical Center, Inc. ("HMFP"). The new facility (the "Cancer Facility") would be owned and operated by DFCI, under its license, and would be located on land owned by BIDMC between the DFCI and BIDMC campuses in Boston, Massachusetts. The clinical collaboration between DFCI, BIDMC, and HMFP, and the construction of the Cancer Facility remain subject to the review and/or approval of certain state and local governmental authorities, including the Massachusetts Department of Public Health, the Massachusetts Health Policy Commission, and the City of Boston. On October 24, 2023, DFCI submitted an application to the Massachusetts Department of Public Health requesting a determination of need approving expenditure of \$1.68 billion to construct and equip the Cancer Facility. The application remains in review.

(2) Summary of Significant Accounting Policies

(a) Principles of Consolidation, Basis of Presentation and Use of Estimates

The accompanying consolidated financial statements have been prepared in accordance with U.S. generally accepted accounting principles and include all majority owned subsidiaries, direct member entities and controlled affiliates, including HMFP. All significant intercompany accounts and transactions have been eliminated in consolidation.

The preparation of consolidated financial statements in conformity with U.S. generally accepted accounting principles requires management to make estimates and assumptions that affect the amounts reported in the financial statements and accompanying notes.

In addition, the System is subject to certain inherent risks specific to the healthcare sector, including the following:

- Dependence on revenues derived from reimbursement by the federal Medicare and state Medicaid programs;
- Government regulation, government budgetary constraints, and proposed legislative and regulatory changes;
- Lawsuits alleging malpractice or other claims; and
- Cybersecurity threats and attacks that negatively affect operations.

Such inherent risks also require the use of certain management estimates in the preparation of the System's consolidated financial statements, and it is reasonably possible that a change in such estimates may occur.

(b) Cash and Cash Equivalents

Cash and cash equivalents include investments in highly liquid debt instruments with a maturity of three months or less when purchased, excluding amounts whose use is limited by arrangements under trust

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(In thousands)

agreements or by donors. Cash equivalents held for investment purposes that are classified as investments and assets held by trustees are not reflected as cash equivalents in the statement of cash flows.

(c) Inventories

Inventories, which are included in other current assets in the accompanying consolidated balance sheets, consisting primarily of pharmaceuticals and patient supplies, are stated at lower of average cost or net realizable value.

(d) Investments and Assets Limited or Restricted as to Use

Investments and assets limited or restricted as to use primarily include assets restricted by donors and assets held by trustees under long-term debt and other agreements.

(e) Investments and Investment Income

Investments are reported at fair value. Fair value is the price that would be received to sell an asset or paid to transfer a liability in an orderly transaction between market participants at the measurement date. See note 5 for a discussion of fair value measurements.

Investment income or loss, including realized and unrealized gains and losses on investments, interest, and dividends as well as the changes in equity interests in limited partnerships are included in the excess of revenues over expenses unless the income is restricted by donor or law. Periodically, the System reviews investments where the market value is substantially below cost, and in cases where the decline is considered to be "other than temporary," an adjustment is recorded as a realized loss and a new cost basis is established.

Certain investments are included in investment pools and pooled investment income and gains and losses are allocated to participating funds based upon the System's respective shares of the pool. See notes 5, 12, and 14.

(f) Uncompensated Care

The System provides care without charge or at amounts less than its established rates to patients who meet certain criteria under its charity care policy. The System grants credit without collateral to patients, most of whom are local residents and are insured under third-party arrangements.

(g) Property and Equipment, net

Property and equipment are stated at cost less accumulated depreciation. Depreciation is computed using the straight-line method over the estimated useful lives of depreciable assets, which range from three to forty years.

Gifts of long-lived assets such as land, buildings, or equipment are recorded at fair market value at the date of donation and reported as net assets without donor restrictions (unless explicit donor stipulations specify how the donated assets must be used) and are excluded from the excess of revenues over expenses. Gifts of long-lived assets with explicit restrictions that specify how the assets are to be used

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September 30, 2024 and 2023

(In thousands)

and gifts of cash or other assets that must be used to acquire long-lived assets are reported as net assets with donor restrictions. Absent explicit donor stipulations about how long these long-lived assets must be maintained, expiration of donor restrictions are reported when the donated or acquired long-lived assets are placed in service.

Property and equipment are reviewed for impairment whenever events or changes in circumstances indicate that the carrying amount of an asset may not be recoverable. If circumstances require a long-lived asset be treated for possible impairment, the System first compares undiscounted cash flows expected to be generated by an asset to the carrying value of the asset. If the carrying value of the long-lived asset is not recoverable on an undiscounted cash flow basis, impairment is recognized to the extent that the carrying amount exceeds its fair value. Fair value is determined through various valuation techniques including discounted cash flow models, quoted market values, and third-party independent appraisals, as considered necessary.

(h) Leases

The System determines if an arrangement is a lease or contains a lease at inception and performs the initial classification and measurement of its right-of-use assets and lease liabilities at the lease commencement date and thereafter if modified. The lease term includes any renewal options that the System is reasonably certain to exercise. For leases with an expected lease term of one year or less, the System applies the short-term lease exemption and does not record a right-of-use asset or lease liability.

The System measures its right-of-use assets and lease liabilities at the lease commencement date based on the present value of the remaining lease payments. BILH has elected the practical expedient to use a risk-free rate for leases when establishing a discount rate to determine the present value of lease payments.

(i) Self-Insurance

The System is self-insured for certain health insurance and workers' compensation benefit programs. Estimated losses and claims are accrued as incurred.

(j) Net Assets With Donor Restrictions

Net assets with donor restrictions include those whose use is limited by donor-imposed stipulations, including some that either expire with the passage of time or can be fulfilled and removed by actions of the System pursuant to those stipulations.

The System has interpreted state law as requiring realized and unrealized gains of net assets restricted in perpetuity to be retained in the net asset with donor restrictions classification until appropriated by the board and expended. State law allows the board to appropriate so much of the net appreciation of restricted net assets as is prudent considering the System's long-and short-term needs, present and anticipated financial requirements, as well as expected total return on its investments, price level trends, and general economic conditions.

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Notes to Consolidated Financial Statements

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(In thousands)

(k) Excess (Deficiency) of Revenues over Expenses

The consolidated statements of operations include the excess (deficiency) of revenues over expenses from operating and nonoperating activities. Operating revenues consists of those items attributable to the care of patients, including contributions, research, sale of intellectual property, and investment income on investments without donor restrictions, which are utilized to provide charity and other operational support. Peripheral activities, including realized gains or losses on sales of securities, unrealized changes in equity interests in limited partnerships, as well as changes in unrealized gains or losses on marketable investments are reported as nonoperating activities.

Changes in net assets without restrictions, which are excluded from the (deficiency) excess of revenues over expenses consistent with industry practice, include contributions of long-lived assets (including assets acquired using contributions which by donor restriction were to be used for the purposes of acquiring such assets), transfers to or from affiliates, and changes in the funded status of employee benefit plans, other than net periodic benefit cost.

(l) Revenue Recognition

The System's patient service revenue is reported at the amount that reflects the consideration to which the System expects to be entitled in exchange for providing patient care. These amounts are due from patients, third-party payors (including managed care payors and government programs), and others and include an estimate of variable consideration for retroactive revenue adjustments due to settlement of audits, reviews, and investigations. Generally, the System bills the patients and third-party payors several days after the services are performed and/or the patient is discharged from the System's facility.

Revenue is recognized as performance obligations are satisfied. Performance obligations are determined based on the nature of the services provided by the System. Revenue for performance obligations satisfied over time is recognized based on actual charges incurred in relation to total expected (or actual) charges. The System believes that this method provides a reasonable representation of the transfer of services over the term of the performance obligation based on the inputs needed to satisfy the obligation. Generally, performance obligations satisfied over time relate to inpatient services. The System measures the performance obligation from admission into the hospital, or commencement of a patient service, to the point when it is no longer required to provide services to that patient, which is generally at the time of discharge or completion of the outpatient services. Patient encounters and related episodes of care and procedures qualify as distinct goods and services, provided simultaneously together with other readily available resources, in a single instance of service, and thereby constitute a single performance obligation for each patient encounter and, in most instances, occur at readily determinable transaction prices. All services provided are expected to result in cash flows and are therefore reflected as net revenue in the consolidated financial statements.

The initial estimate of the transaction price is determined by reducing the standard charge by any contractual adjustments, discounts, and implicit price concessions. The estimates of contractual adjustments and discounts are based on contractual agreements, discount policies and historical cash collection experience. Differences between standard charges and estimated transaction price are

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September 30, 2024 and 2023

(In thousands)

generally recorded as adjustments to patient service revenue in the period of the change and are accrued on an estimated basis in the period the related services are rendered and adjusted in future periods as final settlements are determined. Adjustments arising from a change in the transaction price were not significant during the years ended September 30, for 2024 or 2023.

Revenues under the traditional fee for service Medicare and Medicaid programs are based primarily on prospective payment systems. Retrospectively determined cost-based revenues under these programs, which were more prevalent in earlier periods, and certain other payments, such as disproportionate share hospital and bad debt expense reimbursement, which are based on our hospitals' cost reports, are estimated using historical trends and current factors. Cost report settlements under these programs are subject to audit by Medicare and Medicaid auditors and administrative and judicial review, and it can take several years until final settlement of such matters is determined and completely resolved. The System records accruals to reflect the expected final settlements on cost reports. For filed cost reports, the accrual is recorded based on those cost reports and subsequent activity. The accrual for periods for which a cost report is yet to be filed is recorded based on estimates of what the System expects to report on the filed cost reports. After the cost report is filed, the accrual may need to be adjusted.

Settlements with third-party payors for retroactive revenue adjustments due to audits, reviews or investigations are considered variable consideration and are included in the determination of the estimated transaction price for providing patient care using the most likely outcome method. These settlements are estimated based on the terms of the payment agreement with the payor, correspondence from the payor and historical settlement activity, including an assessment to ensure that it is probable that a significant reversal in the amount of cumulative revenue recognized will not occur when the uncertainty associated with the retroactive adjustment is subsequently resolved. Estimated settlements are adjusted in future periods as adjustments become known, or as years are settled or are no longer subject to such audits, reviews and investigations.

The System is not aware of any material claims, disputes, or unsettled matters with any payors that would affect revenues that have not been adequately provided for and disclosed in the accompanying consolidated financial statements. Because the laws, regulations, instructions and rule interpretations governing Medicare and Medicaid reimbursement are complex, subject to interpretation and can change frequently, the estimates recorded could change by material amounts.

Consistent with the System's mission, care is provided to patients regardless of their ability to pay. The System has determined it has provided implicit price concessions to uninsured patients and patients with other uninsured balances (e.g., copays and deductibles). The implicit price concessions included in estimating the transaction price represent the difference between amounts billed to patients and the amounts the System expects to collect based on its collection history with those patients. Patients who meet the System's criteria for charity care are provided care without charge or at amounts less than established rates. The System has determined that it has provided sufficient implicit price concessions for these accounts. Price concessions, including charity care, are not reported as revenue.

**BETH ISRAEL LAHEY HEALTH, INC.
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Notes to Consolidated Financial Statements

September 30, 2024 and 2023

(In thousands)

Patients who are covered by third-party payors are responsible for related co-pays, co-insurance and deductibles, which vary in amount. The System estimates the transaction price for patients with co-pays, co-insurance and deductibles and for those who are uninsured based on historical collection experience and current market conditions. The discount offered to certain uninsured patients is recognized as a contractual allowance, which reduces net operating revenues at the time the self-pay accounts are recorded. The uninsured patient accounts, net of contractual allowances recorded, are further reduced to their net realizable value at the time they are recorded through implicit price concessions based on historical collection trends for self-pay accounts and other factors that affect the estimation process. Although outcomes vary, the System's policy is to attempt to collect amounts due from patients, including co-pays, co-insurance and deductibles due from patients with insurance, at the time of service while complying with all federal and state statutes and regulations.

Other revenue includes contributions and net assets released from restrictions as well as cafeteria and parking income. Additionally, pharmacy sales and other contracts related to health care services are included in other revenue and consist of contracts which vary in duration and in performance. Revenue is recognized when the performance obligations identified within the individual contracts are satisfied and collections are probable. Other revenue for the years ended September 30, 2024 and 2023 also included funding received from federal and state sources related to the COVID-19 pandemic amounting to \$17,500 and \$6,100, respectively.

(m) Donations

Unconditional promises to give cash and other assets to the System are reported at fair value at the date the promise is received. Conditional promises to give and indications of intentions to give are reported at fair value at the date the gift is received or the conditional promise becomes unconditional.

Gifts are reported as net assets with donor restrictions if they are received with donor stipulations that limit the use of the donated assets. When a donor restriction expires, such as when a stipulated time restriction ends or purpose restriction is accomplished, the net assets are reclassified as net assets without donor restrictions and recorded as net assets released from restrictions (which are included with other revenue or as direct additions to net assets, if for property and equipment).

(n) Debt Issuance Costs

Debt issuance costs, original issue discounts and premiums are amortized over the period the related obligation is outstanding, generally using the effective interest method. Debt issuance costs are presented as an offset to the related debt in the consolidated balance sheets.

(o) Research Grants and Contracts

Revenue related to research grants and contracts is recognized under cost reimbursement contracts as the related costs are incurred. Indirect costs relating to certain government grants and contracts are reimbursed at fixed rates negotiated with the government agencies. Amounts received in advance of incurring the related expenditures are recorded as unexpended research grants and are included within accounts payable and accrued expenses in the consolidated balance sheets.

**BETH ISRAEL LAHEY HEALTH, INC.
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Notes to Consolidated Financial Statements

September 30, 2024 and 2023

(In thousands)

(p) Professional Liability

The System insures the majority of its professional liability risks on a claims-made basis in cooperation with several other Harvard-affiliated healthcare organizations through Controlled Risk Insurance Company ("CRICO"), a captive insurance company, of which the System holds an 11.1% ownership interest. The System insures a portion of its professional liability risks on a claims-made basis through Lahey Clinic Insurance Company, a wholly owned affiliate captive reinsurance company.

The System maintains a program of self-insurance to cover professional liability claims incurred but not reported to the captive insurance companies. The estimated amount of accrued unasserted claims has been determined by consulting actuaries on a discounted basis using interest rates of 3.0% for the years ended September 30, 2024 and 2023.

(q) Income Tax Status

Beth Israel Lahey Health, Inc., which serves as the parent of the System, has been determined by the Internal Revenue Service to be an organization described under Internal Revenue Code (the "Code") Section 501(c)(3) and, therefore, is exempt from federal income taxes on related income pursuant to Section 501(a) of the Code. The Internal Revenue Service has also determined that the other entities in the System, excluding its for-profit subsidiaries, qualify as organizations described in Section 501(c)(3) of the Code, meet the Code's requirements under Section 509(a), and therefore are exempt from federal income taxes on related income pursuant to Section 501(a) of the Code. Accordingly, no provision has been recorded for income taxes in the accompanying consolidated financial statements.

The System recognizes the effect of income tax positions only if those positions are more likely than not of being sustained. Recognized income tax positions are measured at the largest amount of benefit that is greater than fifty percent likely to be realized upon settlement. Changes in measurement are reflected in the period in which the change in judgment occurs. The System did not recognize the effect of any income tax positions for the years ended September 30, 2024 and 2023, respectively.

(r) Fair Value of Financial Instruments

The carrying amount of cash and cash equivalents, patient accounts receivable, accounts payable and accrued expenses and estimated settlements with third-party payors approximate fair value because of the short maturity of these instruments.

(s) Recently Adopted Accounting Pronouncement

In June 2016, the Financial Accounting Standards Board ("FASB") issued Accounting Standards Update ("ASU") No. 2016-13, Financial Instruments—Credit Losses (Topic 326), Measurement of Credit Losses on Financial Instruments. Under the ASU, the methodology and accounting for estimating credit losses is standardized across loans and other financial instruments, including receivables. Patient accounts receivable recorded net of implicit price concessions and contributions receivable are outside of the scope of the standard. The System adopted ASU No. 2016-13 for the fiscal year ended September 30, 2024 with no material impact on the consolidated financial statements.

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(In thousands)

(t) Subsequent Events

The System considers events or transactions that occur after the consolidated balance sheet date, but before the consolidated financial statements are issued, to provide additional evidence relative to certain estimates or to identify matters that require additional disclosure. These consolidated financial statements were issued on December 17, 2024 and subsequent events have been evaluated through that date.

(3) Community Service and Uncompensated Care

(a) Community Benefits

The System works in collaboration with its six licensed and affiliated community health centers, local public health officials, community-based organizations, and community residents to conduct community health needs assessments and develop appropriate strategies to address and prioritize needs. The System's Community Benefits program is focused primarily on historically underserved populations and is designed to maintain and improve the health status of community members as well as to ensure that the System is welcoming and provides culturally and linguistically appropriate care and services.

The System's Community Benefits priorities focus on programs and strategies related to Access to Care, Social Determinants of Health, Chronic Disease Management and Risk Factors, and Behavioral Health. Programs and strategies are focused on cohorts experiencing the greatest disparities including youth/adolescents, older adults, low resource individuals and families, racially and ethnically diverse/non-English speakers and the Lesbian, Gay, Bisexual, Transgender and Queer or Questioning ("LGBTQ") community.

The System provides, by hospital license, annual reports describing its community benefit activities to the Massachusetts Attorney General's office. The reports summarize progress made during the past year as well as objectives and initiatives for the upcoming year. The System's most recent reports for fiscal year 2023 include descriptions of community programs provided by the System at a cost of approximately \$98,425 with an additional \$17,044 in leveraged resources (i.e., grants) and \$47,979 in charity care. The System is in the process of compiling its annual reports for fiscal year 2024.

(b) Charity Care

The System provides care without charge or at amounts less than its established rates to patients who meet certain criteria under its charity care policies. Because the System does not pursue collection of amounts determined to qualify as charity care, they are not reported as revenue except to the extent reimbursed by the Massachusetts Health Safety Net Trust ("Health Safety Net Trust").

The System also makes payments to the Health Safety Net Trust to support the delivery of charity care to patients throughout Massachusetts. Payments to the Health Safety Net Trust primarily support Massachusetts' Medicaid and Children's Health Insurance Programs ("Medicaid Programs"). These payments are reported as a component of supplies and other expenses in the consolidated statements of operations.

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(In thousands)

The System's net cost of charity care, determined using a ratio of cost to charge, including care for services provided to nonpaying patients and including payments to and receipts from the Health Safety Net Trust, was as follows:

	<u>2024</u>	<u>2023</u>
Charity care, at cost	\$ 46,378	40,680
Payments to Health Safety Net Trust and Medicaid Programs	96,604	105,775
Less:		
Payments related to Medicaid Programs	(87,284)	(66,919)
Payments from Health Safety Net Trust	<u>(17,364)</u>	<u>(18,916)</u>
Net charity care	\$ <u>38,334</u>	<u>60,620</u>

The payments to the Health Safety Trust comprise (a) the original assessment of \$89,592 for both of the years ended September 30, 2024 and 2023; and (b) additional shortfall assessments of \$7,012 and \$16,183 for the years ended September 30, 2024 and 2023, respectively. The System's original assessment is comprised of \$22,671 related to charity care and \$66,921 related to the Medicaid Programs, with all of the shortfall assessments being fully related to charity care.

(c) Other Uncompensated Care

The System provides care to patients who participate in other programs designed to support low-income families, including the Medicaid program, which is jointly funded by federal and state governments. The Massachusetts Health Reform Law provided an initiative for expansion of Medicaid coverage to greater populations and for enrollment of uninsured patients in other insurance programs.

Payments from Medicaid and other programs which insure low-income populations do not cover the cost of services provided. In aggregate, the cost of care provided by the System for such services exceeded reimbursement by \$304,587 and \$209,654 for the years ended September 30, 2024 and 2023, respectively.

The System also provides care to patients who participate in the Medicare program, the federally sponsored health insurance program for elderly or disabled patients. Payments to the System for those services also do not cover the costs of services provided. In aggregate, the cost of care provided by the System for such services exceeded reimbursement by \$649,381 and \$456,416 for the years ended September 30, 2024 and 2023, respectively.

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(In thousands)

(4) Patient Service Revenue

The following table reflects patient service revenue from third-party payors, government subsidies and other (including uninsured patients):

	<u>2024</u>	<u>2023</u>
Medicare	\$ 3,058,751	2,691,917
Medicaid	706,930	747,371
Blue Cross	1,448,372	1,247,603
Commercial insurance and managed care	1,700,200	1,562,203
Patient	41,545	64,172
Other	<u>145,601</u>	<u>126,058</u>
Total patient service revenue	\$ <u>7,101,399</u>	<u>6,439,324</u>

(5) Investments and Assets Limited or Restricted as to Use

The System invests in various investment securities. Investment securities are exposed to various risks such as interest rate, market, and credit risks. Due to the level of risk associated with certain investment securities, it is at least reasonably possible that changes in the values of investment securities will occur in the near term and those changes could materially affect the amounts reported in the consolidated financial statements.

Fair value represents the price that would be received upon the sale of an asset or paid upon the transfer of a liability in an orderly transaction between market participants as of the measurement date. Financial instruments that are measured and reported at fair value are classified and disclosed in one of the following categories:

- Level 1 – quoted prices (unadjusted) in active markets that are accessible at the measurement date for assets or liabilities.
- Level 2 – observable prices that are based on inputs not quoted in active markets, but corroborated by market data.
- Level 3 – unobservable inputs are used when little or no market data is available. Significant professional judgment is used in determining the fair value assigned to such assets or liabilities.

Following is a description of the valuation methodologies used for assets at fair value:

Cash equivalents: Consist of money market funds, which are valued at \$1 per share, which is the price the shares trade at in an active market.

Equities: Valued at the closing price reported on an active market on which the individual securities are traded.

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(In thousands)

Private equities, credit related, and real assets: The estimation of fair value of investments in investment companies for which an investment does not have a readily determinable value is made using the net asset value ("NAV") per share or its equivalent as a practical expedient.

Fixed income: The accounts invest principally in fixed income instruments and debt instruments. Account investments are primarily valued using market quotations or prices obtained from independent pricing sources which may employ various pricing methods to value the investments including matrix pricing.

The System owns interests in alternative investment funds that are generally reported at the NAV reported by the fund managers, unless the fund has a readily determinable fair value, which is used as a practical expedient to estimate the fair value of the System's interest therein, unless it is probable that all or a portion of the investment will be sold for an amount different from NAV. Such valuations are determined by fund managers and generally consider variables such as operating results, comparable earnings multiples, projected cash flows, recent sales prices, and other pertinent information, and may reflect discounts for the illiquid nature of certain investments held. As of September 30, 2024 and 2023, respectively, the System had no plans or intentions to sell investments at amounts different from NAV.

The preceding methods may produce a fair value calculation that may not be indicative of net realizable value or reflective of future fair values. Furthermore, although the System believes its valuation methods are appropriate and consistent with other market participants, the use of different methodologies or assumptions to determine the fair value of certain financial instruments could result in a different fair value measurement at the reporting date.

The following table set forth the System's financial assets that were accounted for at fair value on a recurring basis as of September 30, 2024. Investments are classified in their entirety based on the lowest level of input that is significant to the fair value measurement and include related strategy, liquidity, and funding commitments:

	NAV or equivalent	Level 1	Level 2	Level 3	Total	Redemption period frequency	Days notice
Investments:							
Domestic equities	\$ 103,710	385,101	—	—	488,811	Daily – Illiquid	1–N/A
Global equities	190,437	418,096	—	—	608,533	Daily – Illiquid	1–N/A
Private equity and venture capital	201,494	—	—	—	201,494	Illiquid	N/A
Absolute return and hedged equity	844,863	89,824	—	—	934,687	Daily – Illiquid	1–N/A
Credit related	43,811	—	—	—	43,811	Illiquid	N/A
Real assets	60,814	105,678	—	—	166,492	Daily – Illiquid	1–N/A
Fixed income	40,595	517,969	11,701	—	570,265	Daily	1
Cash equivalents	—	312,862	—	—	312,862	Daily	1
Other	18,128	38,640	1,743	56,528	115,039	Daily – Illiquid	1–N/A
Total	\$ <u>1,503,852</u>	<u>1,868,170</u>	<u>13,444</u>	<u>56,528</u>	<u>3,441,994</u>		

**BETH ISRAEL LAHEY HEALTH, INC.
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(In thousands)

Global equities of \$608,533 noted in the table above are comprised of the following strategies: 62.6% Developed Markets, 22.2% Global Value and 15.2% Emerging Markets (actively managed).

Absolute return and hedged equity of \$934,687 noted in the table above are comprised of the following strategies: 1.6% Event Driven, 46.6% Global Long-Short, 0.2% Relative Value, 33.6% Open Mandate and 18.0% Credit Distressed.

The following table presents additional information about the changes in Level 3 assets measured at fair value for the year ended September 30, 2024:

	Fair-value measurements using significant unobservable inputs			
	Beginning balance	Distributions	Unrealized gains	Ending balance
Other	\$ 55,196	(197)	1,529	56,528

The following table describes the redemption frequency or liquidity of investments as of September 30, 2024:

		Daily	Weekly/ monthly	Quarterly	Annually	Greater than one year and illiquid	Total
Domestic equities	\$	385,099	—	69,716	11,834	22,162	488,811
Global equities		183,386	319,258	91,789	—	14,100	608,533
Private equity and venture capital		—	—	—	—	201,494	201,494
Absolute return and hedged equity		120,971	3,541	401,830	162,766	245,579	934,687
Credit related		—	—	—	—	43,811	43,811
Real assets		105,678	—	—	—	60,814	166,492
Fixed income		570,265	—	—	—	—	570,265
Cash equivalents		312,862	—	—	—	—	312,862
Other		37,982	6,372	7,913	15,454	47,318	115,039
Total	\$	<u>1,716,243</u>	<u>329,171</u>	<u>571,248</u>	<u>190,054</u>	<u>635,278</u>	<u>3,441,994</u>

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The following table set forth the System's financial assets that were accounted for at fair value on a recurring basis as of September 30, 2023. Investments are classified in their entirety based on the lowest level of input that is significant to the fair value measurement and include related strategy, liquidity, and funding commitments:

	NAV or equivalent	Level 1	Level 2	Level 3	Total	Redemption period frequency	Days notice
Investments:							
Domestic equities	\$ 97,093	297,741	—	—	394,834	Daily – Illiquid	1–N/A
Global equities	121,223	354,380	—	—	475,603	Daily – Illiquid	1–N/A
Private equity and venture capital	171,355	—	—	—	171,355	Illiquid	N/A
Absolute return and hedged equity	740,143	83,692	—	—	823,835	Daily – Illiquid	1–N/A
Credit related	40,371	—	—	—	40,371	Illiquid	N/A
Real assets	55,617	93,061	—	—	148,678	Daily – Illiquid	1–N/A
Fixed income	—	508,273	175,141	—	683,414	Daily	1
Cash equivalents	—	177,413	—	—	177,413	Daily	1
Other	18,624	29,115	—	55,196	102,935	Daily – Illiquid	1–N/A
Total	<u>\$ 1,244,426</u>	<u>1,543,675</u>	<u>175,141</u>	<u>55,196</u>	<u>3,018,438</u>		

Global equities of \$475,603 noted in the table above are comprised of the following strategies: 62.8% Developed Markets, 17.7% Global Value and 19.5% Emerging Markets (actively managed).

Absolute return and hedged equity of \$823,835 noted in the table above are comprised of the following strategies: 1.7% Event Driven, 51.6% Global Long-Short, 0.2% Relative Value, 28.0% Open Mandate and 18.5% Credit Distressed.

The following table presents additional information about the changes in Level 3 assets measured at fair value for the year ended September 30, 2023:

	Fair-value measurements using significant unobservable inputs			
	Beginning balance	Distributions	Unrealized gains	Ending balance
Other	\$ 47,896	(454)	7,754	55,196

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(In thousands)

The following table describes the redemption frequency or liquidity of investments as of September 30, 2023:

		Daily	Weekly/ monthly	Quarterly	Annually	Greater than one year and illiquid	Total
Domestic equities	\$	297,740	—	67,997	13,114	15,983	394,834
Global equities		163,780	229,219	68,286	—	14,318	475,603
Private equity and venture capital		—	—	—	—	171,355	171,355
Absolute return and hedged equity		110,493	40,826	327,152	132,171	213,193	823,835
Credit related		—	—	—	—	40,371	40,371
Real assets		93,061	—	—	—	55,617	148,678
Fixed income		673,432	9,982	—	—	—	683,414
Cash equivalents		177,413	—	—	—	—	177,413
Other		29,115	5,680	8,652	4,292	55,196	102,935
Total	\$	<u>1,545,034</u>	<u>285,707</u>	<u>472,087</u>	<u>149,577</u>	<u>566,033</u>	<u>3,018,438</u>

Commitments

Private equity and venture capital, absolute return and hedged equity, credit related, and real asset investments are generally made through limited partnerships. Under the terms of these agreements, the System is obligated to remit additional funding periodically as capital or liquidity calls are exercised by the manager. These partnerships have a limited existence, generally up to ten years. As of September 30, 2024, the System projects that the commitments to these partnerships will be exercised by the managers as follows:

Fiscal year		Private equity and venture capital	Credit related	Real assets	Total
2025–2034	\$	90,472	4,054	30,844	125,370

The System is a partner in the BILH Investment Partnership, which holds and manages certain investments on behalf of the System. The System contributed \$11,671 and \$13,479 to the Partnership for 2024 and 2023, respectively and withdrew \$39,590 and \$5,210 from the Partnership for 2024 and 2023, respectively. The total market value of the System's investments held by the Partnership was \$1,527,846 and \$1,322,219 as of September 30, 2024 and 2023, respectively.

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(In thousands)

(6) Liquidity

Financial assets available for general expenditure within one year of the balance sheet date, consist of the following:

	September 30	
	2024	2023
Cash and cash equivalents	\$ 328,294	175,622
Current investments	2,225,877	2,127,730
Patient accounts receivable	924,776	722,681
Contributions and other receivables	211,702	258,911
Estimated appropriation of endowment assets	21,796	18,187
	\$ 3,712,445	3,303,131

The System has the ability to structure its financial assets to be available, as its general expenditures and their obligations come due. Cash in excess of daily requirements are invested in short-term investments.

(7) Contributions Receivable

Contributions receivable, which are included within assets held for specific purposes and endowments in the consolidated balance sheets, consisted of the following:

	September 30	
	2024	2023
Due in less than one year	\$ 35,281	46,808
Due in one to five years	58,363	38,466
Due in more than five years	36,859	22,858
	130,503	108,132
Less:		
Discount to present value at rates ranging from 0.25% to 5.2%	(14,030)	(9,024)
Allowance for uncollectible amounts	(15,390)	(16,183)
	\$ 101,083	82,925

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(In thousands)

(8) Property and Equipment, Net

Property and equipment, net consisted of the following:

	September 30	
	2024	2023
Land	\$ 185,767	185,350
Buildings and improvements	3,732,339	3,594,776
Equipment	3,978,526	3,536,303
Construction in progress	158,242	287,661
	<u>8,054,874</u>	<u>7,604,090</u>
Less accumulated depreciation	<u>(5,187,298)</u>	<u>(4,891,559)</u>
	<u>\$ 2,867,576</u>	<u>2,712,531</u>

During fiscal year 2024, the System began a project to implement EPIC, an electronic medical records software system, throughout the BILH entities. Capital expenditures incurred related to that project are recorded within Equipment for entities that completed implementation during fiscal year 2024 and within Construction in progress for entities where implementation was on-going as of September 30, 2024. During fiscal year 2023, the System completed construction of a new 10-story, approximately 345,000 square foot inpatient building on the BIDMC campus, which opened on April 2, 2023.

For additional discussion of capital projects at facilities of the System included within Construction in progress, see note 9.

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(In thousands)

(9) Long-Term Debt and other Indebtedness

Long-term debt and other indebtedness consisted of the following:

	September 30	
	2024	2023
Fixed-rate debt:		
Massachusetts Development Finance Agency Revenue Bonds:		
Lahey Issue, Series F	\$ 198,715	206,515
CareGroup Issue, Series H	56,665	97,020
CareGroup Issue, Series I	159,300	167,320
CareGroup Issue, Series J	415,640	418,848
BILH Issue, Series K	142,995	148,840
BILH Issue, Series L	500,000	500,000
BILH Issue, Series M	400,000	—
Variable-rate debt:		
Massachusetts Development Finance Agency Revenue Bonds:		
Winchester Issue, Series F	19,745	21,480
New Hampshire Health and Education Facilities Authority Revenue Bonds:		
Exeter Issue, Series 2012	17,680	19,375
Exeter Issue, Series 2003	9,585	10,365
Exeter Issue, Series 2001B	—	1,095
Non-amortizing debt:		
Federal Home Loan Bank (FHLB), Nike Project	—	400
Massachusetts Housing Partnership Fund Board and Community		
Economic Development Assistance Corporation, Nike Project	751	697
Other	664	1,716
Short-term borrowings:		
Taxable Commercial Paper	118,300	—
Debt issuance costs	(14,056)	(12,185)
Net unamortized original issue premiums, net of discounts	116,765	86,690
	<u>2,142,749</u>	<u>1,668,176</u>
Less current portion and short-term borrowings	(159,018)	(55,996)
	<u><u>\$ 1,983,731</u></u>	<u><u>1,612,180</u></u>

Certain long-term indebtedness, as enumerated below, is secured by a gross receipts pledge of the Members of the Obligated Group pursuant to a Master Trust Indenture. The following entities are the Members of the Obligated Group:

- Beth Israel Deaconess Medical Center, Inc.
- Lahey Clinic Foundation, Inc.

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(In thousands)

- Lahey Clinic, Inc.
- Lahey Clinic Hospital, Inc.
- Mount Auburn Hospital
- Northeast Hospital Corporation
- Winchester Hospital
- Beth Israel Deaconess Hospital – Plymouth, Inc.
- New England Baptist Hospital
- Anna Jaques Hospital
- Beth Israel Deaconess Hospital – Milton, Inc.
- Beth Israel Deaconess Hospital – Needham, Inc.
- Medical Care of Boston Management Corporation
- Mount Auburn Professional Services, Inc.
- Exeter Hospital, Inc.

The Sole Member is the Obligated Group Representative but is not a Member of the Obligated Group itself. The Obligated Group's net assets without restriction represented 91.9% and 84.5% of the System's combined net assets without donor restrictions as of September 30, 2024 and 2023, respectively. Exeter Hospital, Inc. joined the Obligated Group effective July 12, 2024, approximately one year after joining the System on July 1, 2023. The Obligated Group is required to maintain a minimum debt service coverage ratio and comply with certain other covenants as specified in the Master Trust Indenture.

Each Obligated Group Member is jointly and severally liable for payments due and payable in connection with the following long-term indebtedness: (i) the Massachusetts Development Finance Agency Revenue Bonds, (ii) the New Hampshire Health and Education Facilities Authority Revenue Bonds (effective July 12, 2024), and (iii) the taxable commercial paper which aggregated \$2,038,624 and \$1,560,023 at September 30, 2024 and 2023, respectively.

Prior to becoming a Member of the Obligated Group, Exeter Hospital debt issuances under the New Hampshire Health and Education Facilities Authority Revenue Bonds, were secured by gross receipts and a mortgage lien on existing and future property, plant and equipment. Exeter Hospital was required to maintain a minimum debt service cover ratio and comply with certain other covenants as specified in its amended and restated Master Trust Indenture, dated September 1, 2021.

In November 2023, the Obligated Group defeased a portion of the Series H-1 bonds in the amount of \$23,250. The System recorded a loss on defeasance in the amount of \$919.

In December 2023, the Obligated Group issued \$400,000 tax-exempt Series M (2023) bonds. Proceeds of the issue are being used to finance various projects at the facilities of the Obligated Group. The Projects consist of a portion of the acquisition and implementation of the EPIC and Workday electronic medical

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record and information systems System-wide, as well as renovations, repairs and acquisition of equipment at the facilities of each of the members of the Obligated Group.

In December 2023, the Obligated Group offered up to \$200,000 in taxable commercial paper notes ("Notes"). The Notes can be issued at a discount in lieu of interest, or bear a market interest rate, with a maturity period not to exceed 270 days. As of September 30, 2024, \$118,300 in Notes were outstanding under the offering and were recorded as short-term borrowings within the consolidated balance sheets. The intent of this issuance is designated for general corporate purposes.

In 2004, NBH received \$300 from the Federal Home Loan Bank to fund a project which included renovations and improvements to property owned by NBH in Topsfield, Massachusetts for the purpose of running residential behavioral health programs. In 2007, the FHLB debt was increased to \$400 and the entire balance was secured by a promissory note. The terms of the debt provide that no payments are due for a period of fifteen years from the completion of the project as long as the property continues to be used for the intended purpose and NBH does not otherwise default. Upon the satisfactory conclusion, the note was deemed satisfied, which occurred in January 2024.

In August 2007, NBH entered into a \$2,800 non-amortizing loan agreement from the Massachusetts Housing Partnership Fund Board and the Community Economic Development Assistance Corporation. The terms of the agreement dictate that no interest will be charged and that the term of the loan may be extended indefinitely, provided that NBH does not default. Default occurs if NBH fails to continue to use the land under the Nike project for its intended use. Should this occur, NBH would be liable for the full amount of the loan. The financing also places limits on the net cash flow that NBH can recognize from the Nike project.

NBH has calculated interest on these mortgages using a rate of 7.75% and reports interest expense and contribution revenue in connection with the loan. Contribution revenue is recorded based on the difference between the fair value of the loans using the calculated interest rate of 7.75% and the stated rate of zero. Each year, NBH recognizes interest expense based on the recorded loan balance and the calculated interest rate. For the amount of recognized interest expense during each period there will also be an offsetting increase to the loan payable, a decrease to net assets with donor restrictions, and an increase to net assets released for restrictions used for operations. As of September 30, 2024, the balance of the loan outstanding was \$751.

As of September 30, 2024 and 2023, the System had in place lines of credit aggregating to \$500,000 and \$500,000, respectively in principal amounts. These lines of credit include various interest rates and payment terms with three different banks and expire in May 2025 (\$250,000 facility), July 2025 (\$170,000 facility), and October 2025 (\$80,000 facility). As of September 30, 2024 and 2023, there were no amounts outstanding against the lines of credit.

Effective October 1, 2024, HMFP has applied for, and has been granted, a line of credit in the amount of \$30,000. The interest rate is a yearly rate equal to the Term SOFR Daily Floating Rate plus 1.00 percentage point. No amounts have been drawn under this line of credit.

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The System's revenue bonds bear interest, mature and are redeemable prior to maturity as follows:

<u>Issue</u>	<u>Interest rate</u>	<u>Maturity</u>	<u>Redemption terms</u>
CareGroup:			
Series H	3.3% to 5.0%	2033	Beginning in 2025 at 100%
Series I	3.0% to 5.0%	2038	Beginning in 2027 at 100%
Series J	5.0 %	2053	Beginning in 2028 at 100%
BILH:			
Series K	4.0% to 5.0%	2041	Beginning in 2029 at 100%
Series L	2.2% to 3.1%	2028, 2051	Currently with conditions
Series M	5.0%	2032, 2034	Beginning in 2032, 50% with conditions
Lahey:			
Series F	5.0%	2045	Currently with conditions
Winchester:			
Series F	Variable, currently 4.0%	2030	Currently with conditions
Exeter:			
Series 2012	Variable, currently 5.0%	2031	Currently with conditions
Series 2003	Variable, currently 5.0%	2033	Currently with conditions
Series 2001B	Variable, 4.5% at September 30, 2023	2023	Matured as of October 1, 2023

Scheduled principal repayments and sinking fund requirements on long-term debt and short-term borrowings for the next five years are as follows:

Year ending September 30:	
2025	\$ 159,018
2026	42,690
2027	44,705
2028	146,695
2029	45,570
Thereafter	<u>1,601,362</u>
	<u>\$ 2,040,040</u>

Interest paid on all outstanding debt amounted to \$81,807 and \$62,772, which includes \$1,466 and \$8,791 of capitalized interest, for the years ended September 30, 2024 and 2023, respectively.

(10) Assets Held by Trustees and other Agreements

Assets held by trustees and other agreements include amounts held in trust under the requirements of various debt and other agreements. The terms of the Revenue Bonds require the establishment of certain

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reserve funds which are held by trustees. These funds, principally comprised of cash equivalents and government securities, are carried at fair market value and are as follows:

	September 30	
	2024	2023
Debt agreements	\$ 1,190	1,190
Capital reimbursement fund	143,374	—
Professional liability trust	49,694	61,614
	<u>\$ 194,258</u>	<u>62,804</u>

(11) Leases

The System's portfolio includes right-of-use assets and lease liabilities for operating leases associated with real estate, clinical, research, and diagnostic equipment and administrative equipment. The operating leases generally have three to ten year terms, with one or more renewal options. The exercise of such lease renewal options is at the System's sole discretion, and to the extent it is reasonably certain that such renewal option will be exercised, the years related to that option are included in the determination of the lease term. Certain leases also include options to purchase the leased assets at a price that either approximates or exceeds fair value.

The majority of leases contain fixed lease payments and lease payments tied to usage which may be subject to variability. The System recognizes rental expense for its operating leases on a straight-line basis over the lease term based on the total fixed lease payments, of which \$119,321 and \$94,296 is included in supplies and other expenses in the consolidated statements of operations for the years ended September 30, 2024 and 2023, respectively. Variable lease payments are recognized as incurred, of which \$48,081 and \$37,264 is included in supplies and other expenses in the consolidated statements of operations for the years ended September 30, 2024 and 2023, respectively.

The components of lease cost are comprised of the following amounts:

	September 30	
	2024	2023
Operating	\$ 119,321	94,296
Short-term	14,160	15,770
Variable	48,081	37,264
Less sublease income	<u>(24,172)</u>	<u>(12,948)</u>
Total lease cost	<u>\$ 157,390</u>	<u>134,382</u>

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The following summarizes additional information related to leases:

	September 30	
	2024	2023
Weighted average remaining lease term (years)	9	7
Weighted average discount rate	1 %	1 %

The following lease payments are expected to be paid for each of the following fiscal years:

Year ended September 30:

2025	\$	99,327
2026		92,108
2027		86,152
2028		72,081
2029		39,582
Thereafter		<u>158,482</u>
Total future minimum lease payments		547,732
Less present value discount		<u>(25,508)</u>
Total lease obligations	\$	<u><u>522,224</u></u>

Supplemental cash flow information related to leases is as follows:

	September 30	
	2024	2023
Operating outflows from operating leases	\$ 104,747	82,993
Operating lease liabilities arising from obtaining right-of-use assets	24,115	103,795

(12) Employee Benefit Plans

The System sponsors several noncontributory defined benefit pension plans and defined contribution plans covering substantially all of its employees.

(a) Defined Benefit Plans

Historically, the Beth Israel Deaconess Medical Center Pension Plan ("BIDMC Plan") covered employees of BIDMC and employees of certain other affiliates; the Lahey Clinic Foundation, Inc. Pension Plan ("Lahey Plan") covered certain employees of Lahey and employees of certain affiliates; the Northeast Hospital Corporation Defined Benefit Pension Plan ("Northeast Plan") covered substantially all of the Northeast employees; the Milton Hospital Pension Plan ("Milton Plan") covered

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employees of Milton; the Anna Jaques Hospital Pension Plan ("AJH Plan") covered employees of AJH; the Beth Israel Deaconess Hospital Plymouth Retirement Plan ("Plymouth Plan") covered employees of Plymouth; and the New England Baptist Hospital Retirement Plan ("NEBH Plan") covered substantially all employees of NEB (collectively referred to as the "Plans"). As of July 1, 2023, the System acquired the Pension plan of EHR ("EHR Plan").

Benefits under the BIDMC Plan were frozen as of January 1, 2013; benefits under the Milton Plan were frozen as of July 1, 2004; benefits under Lahey Plan were frozen as of December 31, 2014; and benefits under the Plymouth Plan were frozen as of September 30, 1996.

On December 1, 2019, the BIDMC Plan, the Milton Plan, and the Northeast Plan were merged into the AJH Plan to create the Beth Israel Lahey Health Defined Benefit Plan I ("BILH DB Plan I"). There were no changes to plan benefits provided to participants as a result of the merger.

On September 1, 2020, BILH DB Plan 1 merged into the NEB Plan to form a new plan that retained the name Beth Israel Lahey Health Defined Benefit Plan I. Also on September 1, 2020, the Lahey Plan merged into the Plymouth Plan to create the Beth Israel Lahey Health Defined Benefit Plan II ("BILH DB Plan II"). There were no changes to plan benefits provided to participants as a result of the mergers.

The EHR Plan has a contributory account balance defined benefit plan which covers substantially all employees of the Hospital and Resources. The EHR Plan's funding policy is to contribute annually at a rate intended to provide for the cost of benefits earned during the year determined in accordance with funding requirements of the Department of Labor. The plan benefits are based on the employee's annual compensation during the term of employment.

The System recognizes the funded status of the Plans, the difference between the fair value of the plan assets and the projected benefit obligation, as an asset or liability in its consolidated balance sheet and recognizes the change in that funded status in the year in which the change occurred through changes in net assets without donor restrictions.

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The measurement date used to determine pension assets and obligations was September 30. The following table sets forth the Plans' funded status and amounts recognized in the consolidated balance sheets:

	September 30	
	2024	2023
Change in benefit obligations:		
Benefit obligation at beginning of period	\$ 1,432,053	1,414,801
Effects of acquisition – EHR	—	85,945
Interest cost	84,686	78,279
Service costs	7,822	5,728
Employee contributions	2,434	645
Benefits paid	(113,779)	(113,369)
Actuarial loss (gain)	160,076	(39,976)
Benefit obligation at end of year	<u>\$ 1,573,292</u>	<u>1,432,053</u>
Change in plan assets:		
Fair value of plan assets at beginning of period	\$ 1,221,460	1,130,934
Effects of acquisition – EHR	—	102,471
Actual return on plan assets	240,747	86,264
Employer contributions	48,201	15,280
Employee contributions	2,434	645
Administrative expenses	(416)	(765)
Benefits paid	(113,779)	(113,369)
Fair value of plan assets at end of year	<u>\$ 1,398,647</u>	<u>1,221,460</u>
Funded status:		
Net pension liabilities at end of year	\$ 174,645	210,593
Included in the Consolidated Balance Sheets as follows:		
Employee benefit plan liabilities	\$ 199,089	232,435
Other assets	<u>(24,444)</u>	<u>(21,842)</u>
	<u>\$ 174,645</u>	<u>210,593</u>

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Amounts not yet reflected in net periodic pension expense and included in the change in net assets without donor restrictions are as follows:

	September 30	
	2024	2023
Net actuarial loss	\$ 362,554	415,264

The estimated amount that will be amortized from net assets without donor restrictions into net periodic pension expense in 2025 is \$15,434.

Net periodic pension expense is comprised of the components listed below:

	September 30	
	2024	2023
Interest costs on projected benefit obligations	\$ 84,687	78,279
Service costs for benefits earned	7,822	5,727
Expected return on plan assets	(64,053)	(53,232)
Amortization of net actuarial losses	16,419	22,173
Net periodic pension expense	<u>\$ 44,875</u>	<u>52,947</u>

The following assumptions were used to determine net periodic pension expense and benefit obligations:

	2024	2023
Weighted average discount rates (pension cost)	6.08 - 6.18%	5.48 - 5.67%
Weighted average discount rates (benefit obligation)	5.01 - 5.05%	6.08 - 6.18%
Expected long-term rates of return on plan assets (pension cost)	6.50 - 7.00%	6.00 - 6.60%
Expected long-term rates of return on plan assets (benefit obligation)	6.40 - 6.80%	6.75 - 7.00%

The methodology for selecting the discount rates for the Plans is to determine the single equivalent yield of a theoretical portfolio of high-quality corporate bonds that would settle each Plan's cash flows based on the expected benefit payments from each Plan as of the measurement date, subject to change at each subsequent measurement date.

The Plans' overall investment objective is to ensure sufficient funds are available to meet the Plans' benefit and reasonable expense obligations. Plan assets will be invested in a prudent manner to meet the obligation of providing benefits to Plan participants and their beneficiaries. In June 2022, a new

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investment policy statement was adopted. Through the Plans' investment policy, the long-term investment objective is to effectively manage the surplus/(deficit) risk between Plan assets and Plan liabilities. An effective hedging strategy may be considered in the investment program to mitigate interest rate risk in the Plan, as the present value of the Plan's liabilities are sensitive to changes in interest rates. The investment program shall consider the potential negative impact of fluctuating interest rates on the surplus/(deficit). In addition, various techniques are utilized to monitor, measure, and manage risk.

The Plans' target allocation is determined taking into consideration the amounts and timing of projected liabilities, the System's funding policies and expected returns on various asset classes. At September 30, 2024 and September 30, 2023, the Plans' target allocation was 40%-60% equity securities, 28%-48% debt securities, and 12%-20% other.

Expected benefit payments for the next five years and total expected benefit payments for the five subsequent years are as follows:

2025	\$	120,537
2026		118,813
2027		118,257
2028		117,385
2029		115,531
2030 through 2034		549,555

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(b) Defined Benefit Plan Fair Value Measurements

The following is a summary of the Plans' investments as of September 30, 2024 at fair value, using the hierarchy of values described in note 5:

	<u>NAV or equivalent</u>	<u>Level 1</u>	<u>Level 2</u>	<u>Total</u>	<u>Redemption period frequency</u>	<u>Days notice</u>
Investments:						
Domestic equities	\$ —	209,329	—	209,329	Daily	1 – 2
Global equities	62,127	334,515	9,973	406,615	Daily	1 – 2
Private equity and venture capital	1,241	—	—	1,241	Illiquid	N/A
Absolute return and hedged equity	26,145	5,828	10,582	42,555	Daily – Illiquid	65 – N/A
Credit related	235	—	—	235	Illiquid	N/A
Real assets	5,350	40,295	—	45,645	Daily – Illiquid	1 – N/A
Fixed income	9,902	608,334	8,027	626,263	Daily	1
Cash equivalents	—	66,764	—	66,764	Daily	1
Total	<u>\$ 105,000</u>	<u>1,265,065</u>	<u>28,582</u>	<u>1,398,647</u>		

Global equities of \$406,615 noted in the table above are comprised of the following strategies: 81% Developed Markets and 19% Emerging Markets (actively managed).

Absolute return and hedged equity of \$42,555 noted in the table above are comprised of the following strategies: 14% Event Driven, 38% Global Long-Short, and 20% Relative Value, 8% Open Mandate and 20% Credit Distressed.

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The following is a summary of the Plans' investments as of September 30, 2023 at fair value, using the hierarchy of values described in note 5:

	<u>NAV or equivalent</u>	<u>Level 1</u>	<u>Total</u>	<u>Redemption period frequency</u>	<u>Days notice</u>
Investments:					
Domestic equities	\$ —	184,427	184,427	Daily	1 – 2
Global equities	—	322,867	322,867	Daily	1 – 2
Private equity and venture capital	86,238	—	86,238	Illiquid	N/A
Absolute return and hedged equity	99,423	30,669	130,092	Daily – Various	1 – 120
Credit related	376	—	376	Various – Illiquid	90 – N/A
Real assets	6,172	34,948	41,120	Daily – Illiquid	1 – N/A
Fixed income	—	393,034	393,034	Daily	1
Other equity securities	—	12	12	Daily	1
Cash equivalents	—	63,294	63,294	Daily	1
Total	\$ <u>192,209</u>	<u>1,029,251</u>	<u>1,221,460</u>		

Global equities of \$322,867 noted in the table above are comprised of the following strategies: 80% Developed Markets and 20% Emerging Markets (actively managed).

Absolute return and hedged equity of \$130,092 noted in the table above are comprised of the following strategies: 3% Event Driven, 82% Global Long-Short, and 2% Relative Value, 6% Open Mandate and 7% Credit Distressed.

(c) Defined Contribution Plans

The System has defined contribution plans under which it makes annual contributions based on specified percentages of annual compensation and employee contributions. The System contributed \$163,321 and \$157,115 to the plans for the years ended September 30, 2024 and 2023, respectively.

(13) Net Assets

Net Assets without Donor Restrictions

Net assets without donor restrictions include investments and board designated assets set aside by the Board for future capital improvements or liquidity, over which the Board retains control and may at its discretion subsequently use for other purposes. Net assets without donor restrictions as of September 30, 2024 and 2023 were \$3,272,270 and \$3,150,221, respectively.

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Net Assets with Donor Restrictions

Net assets with donor restrictions include those whose use is limited by donor-imposed stipulations, including some that either expire with the passage of time or can be fulfilled and removed by actions of the System pursuant to those stipulations. Additionally, net assets with donor restrictions include those whose use is limited by donor – imposed stipulations that neither expire with the passage of time nor can be fulfilled or otherwise removed by the actions of the System. Investment earnings from net assets with donor restrictions may be donor – restricted for capital or operating needs depending upon the original intent of the donor.

Net assets are released from donor restriction by incurring expenses satisfying the restricted purposes or by occurrence of other events specified by donors. Net assets releases from restrictions and used for operations are recorded in net assets released from restriction. Net assets released from restriction and used for capital purposes are recorded as change in net assets without donor restrictions in the consolidated statements of change in net assets. Net assets with donor restrictions are restricted for the following purposes:

		September	
		2024	2023
Subject to expenditure for specific purpose:			
Capital improvements	\$	196,976	70,705
Research and teaching		456,453	426,213
Clinical		<u>318,425</u>	<u>321,503</u>
Total	\$	<u><u>971,854</u></u>	<u><u>818,421</u></u>

(14) Endowment

The System's endowment funds consist of funds that have been established to provide funding for construction and equipment purchases, as well as funding for healthcare services and research. The System's endowment funds represent a component of the System's total net assets with donor restrictions. These funds are invested by the System. Net assets associated with endowment funds are classified and reported based upon the existence or absence of donor-imposed restrictions.

(a) Relevant Law

The Massachusetts Uniform Prudent Management of Institutional Funds Act ("UPMIFA") permits the governing board to exercise its discretion in determining the appropriate level of expenditures from a donor-restricted endowment fund in accordance with a set of guidelines about what constitutes prudent spending. UPMIFA permits the System to appropriate for expenditure or accumulate so much of an endowment fund as the System determines to be prudent for the uses, benefits, purposes, and duration for which the endowment fund is established. Seven criteria are to be used to guide the System in its yearly expenditure decisions: 1) duration and preservation of the endowment fund; 2) the purposes of the System and the endowment fund; 3) general economic conditions; 4) effect of inflation

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or deflation; 5) the expected total return from income and the appreciation of investments; 6) other resources of the System; and 7) the investment policy of the System.

The System classifies as net assets with donor restrictions (a) the original value of gifts donated to the endowment, (b) the original value of subsequent gifts to the endowment, and (c) accumulations to the endowment made in accordance with the direction of the applicable donor gift instrument at the time the accumulation is added to the fund. The investment income earned on the accumulations to the restricted endowment funds is classified based on donor stipulations as net assets with donor restrictions until the donor-imposed restrictions have been met.

Endowment net assets with donor restrictions composition by type of fund consists of the following:

		September 30	
		2024	2023
Donor-restricted endowment funds:			
Research and teaching	\$	279,590	244,236
Clinical		235,919	205,613
Total endowed net assets	\$	515,509	449,849

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Changes in endowment net assets with donor restrictions:

	With donor restrictions
Endowment net assets at September 30, 2022	\$ 395,467
Investment income, net	43,665
Contributions	13,333
Appropriation of endowment assets for expenditure	(18,187)
Effects of acquisition – EHR	25,059
Other	(9,488)
Endowment net assets at September 30, 2023	449,849
Investment income, net	77,033
Contributions	15,589
Appropriation of endowment assets for expenditure	(21,796)
Other	(5,166)
Endowment net assets at September 30, 2024	\$ <u>515,509</u>

(b) Funds with Deficiencies

From time to time, the fair value of assets associated with individual donor-restricted endowment funds may fall below their original contributed value. There were no such deficiencies as of September 30, 2024 and 2023, respectively, or such deficiencies were temporary and immaterial.

(c) Return Objectives and Risk Parameters

The System has adopted investment and spending policies for endowment assets that attempt to provide a predictable stream of funding to programs supported by its endowment while seeking to maintain the purchasing power of the endowment assets. Endowment assets include those assets of donor-restricted funds that the System must hold in perpetuity or for a donor-specified period. The primary investment objective of the management of the endowment fund is to maintain and grow the fund's real value by generating average annual real returns that meet or exceed the spending rate, after inflation, management fees and administrative costs. Consistent with this goal, the Board of Trustees and the Investment Committee intend that the endowment fund be managed with an intention to maximize total returns consistent with prudent levels of risk and reduce portfolio risk through asset allocation and diversification.

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(d) Strategies Employed for Achieving Objectives

To satisfy its long-term rate-of-return objectives, the System relies on a total return strategy in which investment returns are achieved through both capital appreciation (realized and unrealized) and current yield (interest and dividends). The asset allocation policy is designed to attempt to achieve diversity among capital markets and within capital markets, by investment discipline and management style. The investment portfolio is designed in light of the endowment's needs for liquidity, preservation of purchasing power and risk tolerances.

The System targets a diversified asset allocation that places emphasis on investments in domestic and international equities, fixed income, real assets, private equity, and hedge funds strategies to achieve its long-term return objectives within prudent risk constraints.

(e) Spending Policy and How the Investment Objectives Relate to Spending Policy

Under the System's current long-term investment spending policy, which is within the guidelines specified under state law, 5%–6% of the average of the fair value of qualifying long-term investments applied to a three-year moving average with a one-year lag is appropriated. The appropriation amounted to \$21,796 and \$18,187 for the years ended September 30, 2024 and 2023, respectively.

In establishing these policies, the System considered the expected return on its endowment and its programming needs. Accordingly, the System expects the current spending policy to allow its endowment to maintain its purchasing power and to provide a predictable and stable source of revenue to the annual operating budget. Additional real growth will be provided through new gifts or excess investment return.

(15) Concentrations of Credit Risk

The System grants credit without collateral to its patients, many of whom are local residents and are insured under third-party payor agreements. Management estimates that patient accounts receivable, net of price concessions, were comprised as follows:

	<u>2024</u>	<u>2023</u>
Medicare	34 %	35 %
Medicaid	10	10
Blue Cross	18	17
Commercial insurance and managed care	25	27
Patient	7	6
Other	6	5
	<u>100 %</u>	<u>100 %</u>

The System maintains its cash accounts at various financial institutions. The System had cash balances of \$148,722 and \$48,142 in uninsured accounts at September 30, 2024 and 2023, respectively.

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The System has not experienced any losses in such accounts and periodically evaluates the creditworthiness of the financial institutions with which it conducts business. Management believes the System is not exposed to any significant credit risk with respect to its cash balances.

(16) Functional Expenses

Total expenses by function consisted of the following for the years ended September 30, 2024 and September 30, 2023:

Year ended September 30, 2024				
	Patient care	Research	General and administrative	Total
Salaries and benefits	\$ 4,489,176	183,806	544,600	5,217,582
Supplies and other expenses	2,365,378	197,137	991,934	3,554,449
Depreciation	171,779	11,156	127,301	310,236
Interest	18,644	—	55,224	73,868
	<u>\$ 7,044,977</u>	<u>392,099</u>	<u>1,719,059</u>	<u>9,156,135</u>
Year ended September 30, 2023				
	Patient care	Research	General and administrative	Total
Salaries and benefits	\$ 3,884,491	156,111	655,780	4,696,382
Supplies and other expenses	1,940,772	166,676	690,131	2,797,579
Depreciation	163,809	9,675	109,162	282,646
Interest	17,074	—	31,693	48,767
	<u>\$ 6,006,146</u>	<u>332,462</u>	<u>1,486,766</u>	<u>7,825,374</u>

Salaries, supplies and other expenses are classified based on the type of service provided. Fringe benefits are allocated based on salary expense. Depreciation is allocated based on clinical space utilization. For the year ended September 30, 2024, the System modified its functional expense allocations in accordance with FASB ASC 958-720-45. As a result, management reallocated certain balances for the year ended September 30, 2023 in the table above for comparative purposes.

(17) Acquisition

Under an affiliation agreement effective July 1, 2023, BILH became the sole corporate member of EHR. No consideration was transferred in connection with the affiliation, which was consummated to allow both institutions to further their common mission of promoting the health of the communities they serve. The affiliation was accounted for using the acquisition method of accounting.

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The following table summarizes the value assigned to the EHR net assets recognized as of the affiliation date:

	EHR July 1, 2023
Assets:	
Current assets	\$ 78,849
Property and equipment, net	91,948
Other assets	<u>223,182</u>
Total assets	<u>393,979</u>
Liabilities:	
Current liabilities	59,333
Other liabilities	<u>66,707</u>
Total liabilities	<u>126,040</u>
Net assets recognized	<u><u>\$ 267,939</u></u>

As a result of the affiliation, the System recognized \$267,939 in its 2023 consolidated statements of operations and changes in net assets, classified as follows:

	Without donor restrictions	With donor restrictions	Total
Effects of EHR affiliation	\$ 242,880	25,059	267,939

(18) Contingencies

The System is a party in various legal proceedings and potential claims arising in the ordinary course of its business. In addition, the healthcare industry as a whole is subject to numerous laws and regulations of federal, state, and local governments. Compliance with these laws and regulations can be subject to future government review and interpretation, as well as regulatory actions unknown or unasserted at the time. Government investigations and allegations concerning possible violations by healthcare providers of regulations could result in the imposition of significant fines and penalties, as well as significant repayments of previously billed and collected revenue from patient services.

**BETH ISRAEL LAHEY HEALTH, INC.
AND AFFILIATES (BIDMC SEFA)**

Supplementary Schedule of Expenditures of Federal Awards

Year ended September 30, 2024

Federal grantor/pass-through grantor/program title	Assistance listing number	COVID	Direct or pass-through identifying number	Passed through to subrecipients	Total Federal Expenditures
Research and Development Programs:					
Department of Defense:					
U.S. Army Medical Command					
Basic and Applied Scientific Research					
The Broad Institute of MIT	12.300		N00014-23-1-2465	\$ —	54,131
<i>Basic and Applied Scientific Research Subtotal</i>				—	<u>54,131</u>
Military Medical Research and Development	12.420		Direct	707,347	7,097,116
Boston Children's Hospital	12.420		W81XWH1910561	—	668
Dana-Farber Cancer Institute	12.420		W81XWH2210705	—	28,401
Dana-Farber Cancer Institute	12.420		W81XWH1810500	—	(35,639)
Dana-Farber Cancer Institute	12.420		W81XWH-17-2-0017	—	330
Masonic Medical Research Laboratory	12.420		W81XWH2110784	—	26,412
Massachusetts General Hospital	12.420		W81XWH2010293	—	54,995
Massachusetts General Hospital	12.420		W81XWH2110986	—	20,275
PathMaker Neurosystems Inc.	12.420		HT94252410407	—	13,187
Philips Research	12.420		W81XWH1810332	—	(56,076)
The Henry M. Jackson Foundation	12.420		W81XWH-18-2-0040	—	124,264
University of British Columbia	12.420		W81XWH-21-1-0300	—	(59,018)
University of Colorado Denver	12.420		ID0720010-301-13	—	(1,021)
Vanderbilt University Medical Center	12.420		W81XWH-20-PRMRP-IIIRA-COV	—	14,841
Vanderbilt University Medical Center	12.420		W81XWH-20-2-0046	—	15,573
<i>Military Medical Research and Development Subtotal</i>				<u>707,347</u>	<u>7,244,308</u>
Uniformed Services University Medical Research Projects					
Brigham and Women's Hospital	12.750		HU0001-19-2-0125	—	26,548
<i>Uniformed Services University Medical Research Projects Subtotal</i>				—	<u>26,548</u>
Defense Advanced Research Projects Agency					
Research and Technology Development					
Columbia University	12.910		D20AC00004	—	305,386
Vanderbilt University Medical Center	12.910		HR0011-18-2-0001	—	1,213,436
<i>Research and Technology Development Subtotal</i>				—	<u>1,518,822</u>
<i>Department of Defense Total</i>				<u>707,347</u>	<u>8,843,809</u>
Department of Energy:					
Los Alamos National Laboratory	81.UNK		89233218CNA000001	—	49,158
<i>Department of Energy Total</i>				—	<u>49,158</u>
Department of Labor:					
Local Veterans' Employment Representative Program					
Harvard T.H. Chan School of Public Health	17.804		36C24E20D0016	—	24,822
<i>Department of Labor Total</i>				—	<u>24,822</u>

**BETH ISRAEL LAHEY HEALTH, INC.
AND AFFILIATES (BIDMC SEFA)**

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Federal grantor/pass-through grantor/program title	Assistance listing number	COVID	Direct or pass-through identifying number	Passed through to subrecipients	Total Federal Expenditures
Department of the Interior:					
National Aeronautic and Space Administration					
Science	43.001		Direct	\$ 78,147	566,517
Florida State University	43.001		80NSSC19K1599	—	(16,513)
<i>Science Subtotal</i>				<u>78,147</u>	<u>550,004</u>
Exploration	43.003		Direct	70,908	230,176
<i>Exploration Subtotal</i>				<u>70,908</u>	<u>230,176</u>
<i>Department of the Interior Total</i>				<u>149,055</u>	<u>780,180</u>
Department of Veterans Affairs:					
Boston Veterans Association - Medical Center Healthcare System	64.UNK		36C24123D0062	—	31,429
<i>Department of Veterans Affairs Total</i>				<u>—</u>	<u>31,429</u>
National Science Foundation:					
Engineering	47.041		Direct	80,107	820,354
Computer and Information Science and Engineering	47.070		Direct	—	113,721
<i>National Science Foundation Total</i>				<u>80,107</u>	<u>934,075</u>
Department of Health and Human Services:					
Agency For Healthcare Research and Quality					
Research on Healthcare Costs, Quality and Outcomes	93.226		Direct	178,715	621,947
Boston Medical Center (BMC)	93.226		R01HS028814	—	42,487
Community Research Initiative	93.226		U1OHA29294	—	(1,268)
Harvard Medical School	93.226		R18HS027282	—	41,113
Harvard T.H. Chan School of Public Health	93.226		1 R01 HS029454-01	—	27,049
Massachusetts General Hospital	93.226		4 T32HP32715-03-02	—	117
Oregon Health & Science University	93.226		5 K12 HS026370	—	181,244
The University of Texas	93.226		5 R18 HS029362	—	153,495
University of Michigan	93.226		R01HS028438	—	17,255
Northeastern University	93.226		R01HS028458	—	11,639
<i>Agency For Healthcare Research and Quality Subtotal</i>				<u>178,715</u>	<u>1,095,078</u>
Biomedical Advanced Research and Development Authority (BARDA)					
BARDA Biodefense Medical Countermeasure Development					
Duke University	93.360		HHSO100201400002I	—	18,197
Pharm-Olam, LLC	93.360		75A50232F33002	—	(4,141)
Pharm-Olam, LLC	93.360		75A50120D00016	—	1,176
<i>Biomedical Advanced Research and Development Authority Subtotal</i>				<u>—</u>	<u>15,232</u>
Centers For Disease Control					
Centers For Disease Control Research					
Vanderbilt University Medical Center	93.U01		200-2016-91801	—	98,447
Vanderbilt University Medical Center	93.U01		75D30122C12914	—	25,235
Vanderbilt University Medical Center	93.U01		75D30122C14944	—	287,537
<i>Centers for Disease Control Research Subtotal</i>				<u>—</u>	<u>411,219</u>

**BETH ISRAEL LAHEY HEALTH, INC.
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Prevention of Disease, Disability, and Death through Immunization and Control of Respiratory and Related Diseases	93.083		Direct	\$ —	28,674
Prevention of Disease, Disability, and Death through Immunization and Control of Respiratory and Related Diseases Subtotal				—	28,674
Centers For Disease Control Subtotal				—	439,893
Food and Drug Administration					
Food and Drug Administration Research	93.103		Direct	—	339,522
Brilliant Corporation	93.103		75F40120F19147	—	159
Cincinnati Children's Hospital Medical	93.103		R01FD007267	—	19,504
Duke University	93.103		75F40120C00179	—	(279)
International Consulting Associates, Inc	93.103		75F40121C00063/0353/21/2627	—	1,808
International Consulting Associates, Inc	93.103		75F40121P00059	—	6
International Consulting Associates, Inc	93.103		75F40122C00159	—	193,389
Medical Device Innovation Consortium	93.103		U01FD006292-06S1	—	263,119
Food and Drug Administration Subtotal				—	817,228
Health Resources and Services Administration					
National Research Service Award in Primary Care Medicine	93.186		Direct	10,558	540,306
Health Resources and Services Administration Subtotal				10,558	540,306
Substance Abuse and Mental Health Service Administration					
Substance Abuse and Mental Health Services Projects of Regional and National Significance					
Massachusetts Department of Mental Health	93.243		SCDMH822018082610000	—	(37,838)
Yale University	93.243		H79 SM081775	—	165,872
Substance Abuse and Mental Health Service Administration Subtotal				—	128,034
National Institutes of Health					
Trans-NIH Research Support	93.310		Direct	—	573,485
Harvard Medical School	93.310		OT2OD032581	—	39,323
Harvard Pilgrim Health Care	93.310		UG3OD035533	—	494,052
Massachusetts General Hospital	93.310		UG3TR002878	—	(3,600)
University of Connecticut	93.310		5U54AG075941-3	—	1,711
University of Florida	93.310		U24AR071113	—	31,493
Trans-NIH Research Support Subtotal				—	1,136,464
National Center for Advancing Translational Sciences	93.350		Direct	66,884	719,317
Harvard Medical School	93.350		K12TR004381	—	197,964
Harvard Medical School	93.350		UL1TR002541	—	(184,617)
Harvard Medical School	93.350		UMTR004408	—	18,673
Harvard Medical School	93.350		UM1TR004408	—	1,357,063
National Center for Advancing Translational Sciences Subtotal				66,884	2,108,400
Research Infrastructure Programs	93.351		Direct	—	3,840
Washington University	93.351		R01OD024917	—	(2,782)
Research Infrastructure Programs Subtotal				—	1,058

**BETH ISRAEL LAHEY HEALTH, INC.
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Federal grantor/pass-through grantor/program title	Assistance listing number	COVID	Direct or pass-through identifying number	Passed through to subrecipients	Total Federal Expenditures
National Institute of Environmental Health Sciences				\$	
Environmental Health	93.113		Direct	225,341	1,129,032
Boston University	93.113		R01ES031606	—	2,875
Harvard T.H. Chan School of Public Health	93.113		P30ES000002	—	(1,159)
<i>National Institute of Environmental Health Sciences Subtotal</i>				<u>225,341</u>	<u>1,130,748</u>
National Human Genome Research Institute					
Human Genome Research	93.172		Direct	—	(2,524)
Boston Children's Hospital	93.172		R21HG012397	—	5,650
The University of North Carolina at Chapel Hill	93.172		R01HG013163	—	25,370
The University of North Carolina at Chapel Hill	93.172		R56HG013163	—	106,956
University of Illinois	93.172		U01HG013275	—	41,648
<i>National Human Genome Research Institute Subtotal</i>				<u>—</u>	<u>177,100</u>
National Center for Complimentary and Integrative Health					
Research and Training in Complementary and Integrative Health	93.213		Direct	507,417	2,662,108
Massachusetts General Hospital	93.213		R33AT009306	—	2,470
Spaulding Rehabilitation Hospital	93.213		R01AT012144	—	66,031
<i>National Center for Complimentary and Integrative Health Subtotal</i>				<u>507,417</u>	<u>2,730,609</u>
National Institute of Mental Health					
Mental Health Research Grants	93.242		Direct	369,809	3,958,547
Boston Veterans Association - Medical Center Healthcare System	93.242		R21MH125242	—	(1,014)
Brown University	93.242		R01MH123556	—	184,785
Harvard Medical School	93.242		R21MH125242	—	33
Harvard Pilgrim Health Care	93.242		R01MH133515	—	91,745
Harvard Pilgrim Health Care	93.242		R21MH136855	—	11,901
Massachusetts General Hospital	93.242		R25MH119857	—	102,664
New York University	93.242		R01MH127631	—	24,334
The McLean Hospital	93.242		P50MH115846	—	25,066
The McLean Hospital	93.242		R01MH114982	—	6,352
The McLean Hospital	93.242		R01MH126000	—	96,054
The University of Texas	93.242		R01MH129185	—	72,708
University of California, Los Angeles	93.242		R01MH123575	—	58,698
University of California, San Diego	93.242		R01MH123641	—	24,534
University of Michigan	93.242		R34MH135799	—	3,130
University of Pennsylvania	93.242		R01MH116884	—	32
Washington University	93.242		R01MH127571	—	15,592
Yale University	93.242		U01MH124639	60,480	740,494
<i>National Institute of Mental Health Subtotal</i>				<u>430,289</u>	<u>5,415,655</u>
National Institute on Alcohol Abuse and Alcoholism					
Alcohol Research Programs	93.273		Direct	23,908	3,121,624
Harvard T.H. Chan School School of Public Health	93.273		K01AA027831	—	16,459
<i>National Institute on Alcohol Abuse and Alcoholism Subtotal</i>				<u>23,908</u>	<u>3,138,083</u>

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Federal grantor/pass-through grantor/program title	Assistance listing number	COVID	Direct or pass-through identifying number	Passed through to subrecipients	Total Federal Expenditures
National Institute on Drug Abuse				\$	
Drug Abuse and Addiction Research Programs	93.279		Direct	—	129,670
Boston Children's Hospital	93.279		U01DA055353	—	38,387
Brown University	93.279		R01DA051849	—	27,934
Drexel University	93.279		R01DA054543	—	14,535
Yale University	93.279		UG1DA015831	—	260,095
<i>National Institute on Drug Abuse Subtotal</i>				—	<u>470,621</u>
National Institute of Biomedical Imaging and Bioengineering					
Discovery and Applied Research for Technological Innovations to Improve Human Health	93.286		Direct	491,550	1,782,574
Brigham and Women's Hospital	93.286		R21EB030173	—	39,281
Florida State University	93.286		R01EB032820	—	227,145
Massachusetts General Hospital	93.286		R21EB032932	—	39,357
Rensselaer Polytechnic Institute	93.286		R01EB005807	16,944	136,342
Tufts University	93.286		R21EB033166	—	(2,293)
Tufts University	93.286		R33EB033166	—	5,330
University of Arkansas	93.286		R01EB031032	—	39,228
University of California, Los Angeles	93.286		R01EB029751	—	10,432
<i>National Institute of Biomedical Imaging and Bioengineering Subtotal</i>				<u>508,494</u>	<u>2,277,396</u>
National Institute on Minority Health and Health Disparities					
Minority Health and Health Disparities Research	93.307		Direct	86,680	1,506,619
Harvard Pilgrim Health Care	93.307		R01MD017816	50,402	203,941
Tufts University	93.307		R01MD016026	—	107,855
<i>National Institute on Minority Health and Health Disparities Subtotal</i>				<u>137,083</u>	<u>1,818,415</u>
National Institute on Nursing Research					
Nursing Research	93.361		Direct	502,843	957,041
Boston College	93.361		R21NR019874	—	(9,864)
Washington University	93.361		U01NR020555	—	18,689
<i>National Institute on Nursing Research Subtotal</i>				<u>502,843</u>	<u>965,866</u>
National Cancer Institute					
Trans-NIH Research Support	93.310		Direct	—	89,919
<i>Trans-NIH Research Support Subtotal</i>				—	<u>89,919</u>
21st Century Cures Act - Beau Biden Cancer Moonshot					
Dana-Farber Cancer Institute	93.353		U24CA224331	—	138,609
Harvard T.H. Chan School of Public Health	93.353		P50CA244433	—	17,354
<i>21st Century Cures Act - Beau Biden Cancer Moonshot Subtotal</i>				—	<u>155,963</u>
Cancer Cause and Prevention Research	93.393		Direct	118,089	1,664,664
Brigham and Women's Hospital	93.393		R01CA258679	—	(22,775)
Brigham and Women's Hospital	93.393		U01CA250476	—	26,423
Cornell University	93.393		R01CA260352	—	58,435
University of California, San Francisco	93.393		R01CA277782	—	57,859
University of Florida	93.393		R01CA240341	—	18,610
<i>Cancer Cause and Prevention Research Subtotal</i>				<u>118,089</u>	<u>1,803,216</u>

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Cancer Detection and Diagnosis Research	93.394		Direct	\$ 48,392	2,575,144
Boston University	93.394		U2CCA271898	—	48,196
Boston University	93.394		R01CA275015	—	30,381
Dana-Farber Cancer Institute	93.394		D43CA260640	—	72,351
Fred Hutchinson Cancer Research Center	93.394		U01CA224255	—	81,674
JBS Science, Inc.	93.394		R44C165312	—	(383)
Massachusetts Institute Of Technology	93.394		R01CA235740	—	80,638
Massachusetts Institute Of Technology	93.394		R01CA249151	—	349,374
Nanoview Diagnostics, Inc.	93.394		R43CA257433	—	(10,352)
Northeastern University	93.394		R01CA218500	—	100,148
University of Florida	93.394		R01CA277817	—	75,197
<i>Cancer Detection and Diagnosis Research Subtotal</i>				<u>48,392</u>	<u>3,402,368</u>
Cancer Treatment Research	93.395		Direct	749,655	5,140,170
Arizona Board of Regents, University of Arizona	93.395		R21CA267071	—	1,691
Dana-Farber Cancer Institute	93.395		UM1CA186709	—	41,554
ECOG-ACRIN Cancer Research Group	93.395		U10CA180820	—	23,927
NRG Oncology Foundation Inc.	93.395		U10C80868	—	199,933
Oregon Health & Science University	93.395		UG1C89974	—	9,115
Oregon Health & Science University	93.395		U10C80888	—	258,116
<i>Cancer Treatment Research Subtotal</i>				<u>749,655</u>	<u>5,674,506</u>
Cancer Biology Research	93.396		Direct	830,316	8,953,737
Brigham and Women's Hospital	93.396		P01CA120964	—	135,581
Brigham and Women's Hospital	93.396		R21CA238053	—	1,638
Brigham and Women's Hospital	93.396		R21CA238651	—	2,212
Brown University	93.396		R01CA263324	—	345,896
Dana-Farber Cancer Institute	93.396		P50CA206963	—	(8,466)
Harvard T.H. Chan School School of Public Health	93.396		R35CA197459	—	33,510
Massachusetts General Hospital	93.396		P01CA069246	—	(22,222)
Massachusetts Institute Of Technology	93.396		R21CA259840	—	54,598
Vanderbilt University Medical Center	93.396		UH3CA241685	—	60,029
Yale University	93.396		U2CCA252979	—	71,141
<i>Cancer Biology Research Subtotal</i>				<u>830,316</u>	<u>9,627,654</u>
Cancer Centers Support Grants	93.397		Direct	3,001,243	3,347,023
Dana-Farber Cancer Institute	93.397		P50CA272390	—	326,189
Dana-Farber Cancer Institute	93.397		P30CA006516	—	406,312
Dana-Farber Cancer Institute	93.397		P50CA127003	12,711	40,792
Dana-Farber Cancer Institute	93.397		P50CA168504	—	23,853
Dana-Farber Cancer Institute	93.397		P50CA265826	—	51,401
Dana-Farber Cancer Institute	93.397		U54CA156732	—	203,167
Harvard Medical School	93.397		P50CA127003	—	(26,223)
<i>Cancer Centers Support Grants Subtotal</i>				<u>3,013,954</u>	<u>4,372,514</u>
Cancer Research Manpower	93.398		Direct	213,562	751,163
<i>Cancer Research Manpower Subtotal</i>				<u>213,562</u>	<u>751,163</u>

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Cancer Control				\$	
Brigham and Women's Hospital	93.399		UG1CA275416	—	45,189
Dana-Farber Cancer Institute	93.399		U10CA180867	—	(566)
Dana-Farber Cancer Institute	93.399		U10CA180868	—	(390)
ECOG-ACRIN Cancer Research Group	93.399		UG1CA189828	—	70,432
<i>Cancer Control Subtotal</i>				—	114,665
<i>National Cancer Institute Subtotal</i>				4,973,969	25,991,968
National Heart, Lung, and Blood Institute					
National Heart, Lung, and Blood Institute - Research					
Westat (TOPMed ACC)	93.UNK		Westat (TOPMed ACC)	—	28,364
<i>National Heart, Lung, and Blood Institute - Research Subtotal</i>				—	28,364
National Center on Sleep Disorders Research	93.233		Direct	1,388,114	5,288,954
Brigham and Women's Hospital	93.233		R01HL164462	—	22,103
Brigham and Women's Hospital	93.233		R01HL146697	—	11,032
Brigham and Women's Hospital	93.233		R01HL155395	—	18,028
<i>National Center on Sleep Disorders Research Subtotal</i>				1,388,114	5,340,117
Cardiovascular Diseases Research	93.837		Direct	5,345,530	21,311,224
Attune Medical	93.837		R44HL158375-01	—	198,059
Boston University	93.837		R01HL142758	—	22,422
Boston University	93.837		R01HL168889	—	232,844
Boston University	93.837		T32HL125232	—	(3,497)
Brigham and Women's Hospital	93.837		R01HL088521	—	38,551
Brigham and Women's Hospital	93.837		R01HL153814	—	20,220
Brigham and Women's Hospital	93.837		T32HL007901	—	(4,793)
Brigham and Women's Hospital	93.837		U01HL130163	—	12,584
Columbia University	93.837		R01HL139837	—	12
Columbia University	93.837		R01HL141823	—	(42)
Duke University	93.837		R01HL127009	—	(1,807)
Duke University	93.837		R01HL146145	—	(8,244)
Dyad Medical, Inc	93.837		R43HL167290-01A1	—	50,000
EICOSIS, LLC	93.837		R43HL164226	—	(17,885)
Harvard T.H. Chan School School of Public Health	93.837		R01HL035464	—	32,292
Kaiser Foundation Research Institute	93.837		R01HL151996	—	16,371
Massachusetts General Hospital	93.837		R01HL133664	—	57,444
Massachusetts General Hospital	93.837		R01HL151838	—	(105,655)
MedStar Research Institute	93.837		U01HL117006-05	—	1,316
New England Research Institutes, Inc.	93.837		U01HL107407	—	1,782
Northwestern University	93.837		UG3HL165065	—	20,061
NuPulseCV, Inc.	93.837		R43HL158342	—	(50,333)
Ohio State University	93.837		R01HL167511	—	330,452
Ohio State University	93.837		UG3HL140144	—	(6,381)
OROX Biosciences, Inc.	93.837		R3HL160367	—	10,836
Platelet Diagnostics LLC	93.837		R41HL150951	—	1,475
The Broad Institute of MIT	93.837		HHSN2682016000341/75N92020F00001	—	(746,184)

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The University of Texas	93.837		R01HL160793	\$ —	147,148
The University of Texas	93.837		R01HL166272	—	23,651
The University of Texas	93.837		R01HL168683	—	36,294
Thomas Jefferson University	93.837		R01HL145392	—	259
Tufts Medical Center	93.837		R01HL155078	—	3,477
Tulane University	93.837		R01HL034594-36A1	—	2,035
University of California, San Francisco	93.837		R01HL150394	—	12,583
University of California, San Francisco	93.837		R01HL151838	—	92,475
University of Colorado Denver	93.837		R01HL136403	—	1,319
University of Massachusetts Lowell	93.837		R01HL166541	—	265,927
University of Mississippi Medical Center	93.837		R01HL133870	—	524
University of Washington	93.837		R01HL144483	—	35,551
University of Washington	93.837		R01HL144510	—	134,702
University of Washington	93.837		R01HL149706	—	57,243
Vanderbilt University Medical Center	93.837		OT2HL156812	—	23,178
Vanderbilt University Medical Center	93.837		R01HL094786	—	21,163
Vanderbilt University Medical Center	93.837		R01HL149948	—	154,096
Yale University	93.837		N0HL137696	—	7,742
<i>Cardiovascular Diseases Research Subtotal</i>				<u>5,345,530</u>	<u>22,432,491</u>
Lung Diseases Research	93.838		Direct	462,041	1,600,000
Albert Einstein College of Medicine	93.838		R61HL162980	—	23,910
Bay State Health	93.838		U01HL123009	—	11,660
Boston Children's Hospital	93.838		R01HL167723	—	171,657
Boston University	93.838		K23HL153482	—	29,297
Boston University	93.838		R01HL151607	—	13,420
Brigham and Women's Hospital	93.838		OT2HL161847	—	738,011
Brigham and Women's Hospital	93.838		R01HL127200	—	175,632
Brigham and Women's Hospital	93.838		R01HL130974	—	132,168
Brigham and Women's Hospital	93.838		R01HL148667	—	123,586
Brigham and Women's Hospital	93.838		R01HL152728	—	375,534
Brigham and Women's Hospital	93.838		R01HL155522	—	18,889
Columbia University	93.838		UH3HL140177	—	1,730
Duke University	93.838		OT2HL156812	—	200,641
Intermountain Healthcare Research	93.838		R01HL144624	—	11,865
Massachusetts General Hospital	93.838		K08HL141694	—	14,905
Massachusetts General Hospital	93.838		OT2HL156812	74,300	79,360
Massachusetts General Hospital	93.838		R01HL152075	—	37,673
Massachusetts General Hospital	93.838		U01HL123009	38,275	353,911
New York University	93.838		OT2HL156812	—	8,413
New York University	93.838		OT2HL161847	17,859	236,799
NorthShore University HealthSystem	93.838		EH175-S5	—	18,689
Northwestern University	93.838		U01HL146408	15,709	231,268
Rhode Island Hospital	93.838		R01HL162954	—	14,252
Society of Critical Care Medicine (SCCM)	93.838		R01HL162954	—	11,938
University of California, Davis	93.838		R01HL148715	—	183,850
Vanderbilt University Medical Center	93.838		OT2HL156812	—	30,004
Vanderbilt University Medical Center	93.838		R01HL146588	—	345,011
<i>Lung Diseases Research Subtotal</i>				<u>608,184</u>	<u>5,194,073</u>

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Blood Diseases and Resources Research	93.839		Direct	\$ 158,741	4,299,241
Boston University	93.839		R01HL164650	—	294,698
Brigham and Women's Hospital	93.839		P01HL171803	—	59,963
Indiana University	93.839		R01HL131093	—	135,762
Kaiser Foundation Research Institute	93.839		R01HL166292	—	29,252
Massachusetts General Hospital	93.839		P01HL131477	46,667	938,306
Medical University of South Carolina	93.839		R01HL156116	—	358,596
New York University	93.839		UH3HL155798	—	2,295
Rutgers State University of New Jersey	93.839		U01HL133817	—	(108)
The University of Texas	93.839		R35HL139424	—	19,014
University of Kentucky	93.839		R35HL150818	—	81,118
<i>Blood Diseases and Resources Research Subtotal</i>				<u>205,408</u>	<u>6,218,137</u>
<i>National Heart, Lung, and Blood Institute Subtotal</i>				<u>7,547,236</u>	<u>39,213,182</u>
National Institute of Arthritis and Musculoskeletal and Skin Diseases					
Arthritis, Musculoskeletal and Skin Diseases Research	93.846		Direct	239,091	1,659,625
Boston University	93.846		R01AR078299	—	36,398
Boston University	93.846		R01AR079489	—	260,227
Boston University	93.846		R01AR081264	—	244,807
Boston University	93.846		R01AR081393	—	39,489
Boston University	93.846		R33AR078093	—	5,620
Boston University	93.846		R56AR075788	—	89
Harvard School of Dental Medicine	93.846		R21AR076687	—	537
Hebrew Rehabilitation Center	93.846		R01AR041398	—	34,951
Hebrew Rehabilitation Center	93.846		R01AR061445	—	(3,884)
Massachusetts General Hospital	93.846		P30AR075042	—	38,054
Massachusetts General Hospital	93.846		P50AR080596	—	360
MyoLex, Inc.	93.846		R44AR079366	—	281,166
Ortholevo, Inc.	93.846		R44AR079326	—	41,620
Osteocentric Technologies, Inc	93.846		R44AR079300	—	(47,694)
University of Alabama at Birmingham	93.846		R01AR080868	—	30,971
University of California, San Francisco	93.846		UH2AR076724	—	(1,386)
University of Virginia	93.846		R01AR082880	—	3,890
<i>National Institute of Arthritis and Musculoskeletal and Skin Diseases Subtotal</i>				<u>239,091</u>	<u>2,624,840</u>
National Institute of Diabetes and Digestive and Kidney Diseases					
Trans-NIH Research Support					
The Broad Institute of MIT	93.310		U24DK112340	—	977,196
<i>Trans-NIH Research Support Subtotal</i>				<u>—</u>	<u>977,196</u>
Diabetes, Digestive, and Kidney Diseases Extramural Research	93.847		Direct	5,564,606	38,655,433
Arizona Board of Regents, University of Arizona	93.847		R01DK132537	—	351,090
Augusta University	93.847		U24DK115255	—	(1,945)
BioSurfaces	93.847		R43DK137616	—	80,979
Boston Children's Hospital	93.847		P30DK034854	(1,402)	400,744

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Boston Children's Hospital	93.847		R01DK048106	\$ —	489,349
Boston Children's Hospital	93.847		R03DK123541	—	(610)
Boston Children's Hospital	93.847		R21DK138394	—	126,686
Boston Children's Hospital	93.847		RC2DK122532	—	51,609
Boston Medical Center (BMC)	93.847		P30DK046200	—	72,225
Boston Medical Center (BMC)	93.847		U01DK133092	—	233,152
Boston Obesity Nutrition Research Center	93.847		P30DK046200	—	6,025
Boston University	93.847		R01DK116021	—	127,565
Brigham and Women's Hospital	93.847		R01DK048873	—	23,229
Brigham and Women's Hospital	93.847		R01DK103046	—	366,253
Brigham and Women's Hospital	93.847		R01DK125786	—	25,928
Brigham and Women's Hospital	93.847		R01DK126685	—	92,748
Brigham and Women's Hospital	93.847		R01DK133864	—	88,899
Brigham and Women's Hospital	93.847		R25DK101398	—	8,937
Brigham and Women's Hospital	93.847		T32DK007529	—	36,803
Brigham and Women's Hospital	93.847		UH3DK114915	—	15,635
Brown University	93.847		U54AG063546	—	65,203
Case Western Reserve University	93.847		U01DK094157	—	328,400
Celdara Medical, LLC	93.847		R44DK117714	—	(476)
Cleveland Clinic Lerner College of Medicine of CWRU	93.847		U01DK114156	—	28,064
Columbia University	93.847		U01DK136523	—	68,541
George Washington University	93.847		U01DK057154	—	13,902
Georgia State University	93.847		R01DK119202	—	408,780
Georgia State University	93.847		R01DK128823	—	175,455
Harvard Clinical & Translational Science	93.847		UL1TR002541	—	(17)
Joan & Sanford I. Weill Medical College	93.847		R01DK103046	—	(64,818)
Massachusetts General Hospital	93.847		P30DK040561	—	74,206
Massachusetts General Hospital	93.847		P30DK135043	—	252,476
Massachusetts General Hospital	93.847		R01DK116999	—	(3,115)
Massachusetts General Hospital	93.847		R01DK124710	—	173,478
Massachusetts General Hospital	93.847		R01DK133847	—	226,539
Massachusetts General Hospital	93.847		R56DK122593	—	38,189
Massachusetts General Hospital	93.847		U01DK140761	—	2,442
Massachusetts General Hospital	93.847		U24DK132733	—	10,878
Massachusetts Institute of Technology	93.847		R01HD105947	—	236,850
Michigan State University	93.847		R01DK120862	—	49,387
Northern California Institute for Research and Education	93.847		R01DK125646	—	54,341
Pacific Northwest National Laboratory	93.847		DK122160	—	53,739
SanaHeal, Inc.	93.847		R44DK137615	—	441,736
Serplus Technology	93.847		R41DK127818	—	13,750
Stanford University	93.847		K12DK133995	—	13,907
Stanford University	93.847		U01DK130060	—	54,791
The Broad Institute of MIT	93.847		1061307	—	4,322
The Broad Institute of MIT	93.847		R01DK102173	—	17
The Broad Institute of MIT	93.847		R01DK129464	—	300,352
The Broad Institute of MIT	93.847		R01DK132900	—	162,150
The Broad Institute of MIT	93.847		U01DK137242	—	99,137
The Scripps Research Institute	93.847		R01DK134609	—	90,772
The University of Texas	93.847		R35DK095072	—	(8,849)
University of Chicago	93.847		R01DK105588	—	118,848

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University of Colorado Denver	93.847		R01DK121995	\$ —	327,482
University of Colorado Denver	93.847		R01DK134706	—	25,605
University of Connecticut	93.847		U54AG075941	—	252,248
University of Maryland	93.847		R01DK00000	—	(10,585)
University of Maryland	93.847		U24DK126110	—	72,457
University of Massachusetts Worcester	93.847		R01DK130498	—	453,551
University of Michigan	93.847		R01DK117639	—	86,569
University of South Florida Board of Trustees	93.847		U01DK106993	—	244,048
University of Washington	93.847		5UC2114886	—	27,125
University of Wisconsin	93.847		5U54DK130065	—	40,328
Vanderbilt University Medical Center	93.847		R01DK108159	—	(31,818)
<i>Diabetes, Digestive, and Kidney Diseases Extramural Research Subtotal</i>				<u>5,563,204</u>	<u>46,221,120</u>
<i>National Institute of Diabetes and Digestive and Kidney Diseases Subtotal</i>				<u>5,563,204</u>	<u>47,198,316</u>
National Institute of Neurological Disorders and Stroke					
Drug Use and Addiction Research Programs	93.279		Direct	<u>7,045</u>	<u>102,134</u>
<i>Drug Use and Addition Research Programs Subtotal</i>				<u>7,045</u>	<u>102,134</u>
Trans-NIH Research Support					
Massachusetts General Hospital	93.310		UF1NS131791	—	3,465
<i>Trans-NIH Research Support Subtotal</i>				—	<u>3,465</u>
Extramural Research Programs in the Neurosciences and Neurological Disorders	93.853		Direct	3,110,859	10,068,783
Boston Children's Hospital	93.853		R01NS088566	—	9,168
Boston Children's Hospital	93.853		R25NS070682	—	112,407
Boston Children's Hospital	93.853		U24NS107183	—	(18,674)
Boston University	93.853		R01NS017950	—	48,673
Brigham and Women's Hospital	93.853		U54NS112107	—	368,068
CND Life Sciences, Inc.	93.853		R44NS127696	—	191,699
Cutaneous Neurodiagnostics	93.853		R44NS117214	—	423
Dystonia Medical Research Foundation	93.853		U54NS116025	—	112
Icahn School of Medicine at Mount Sinai	93.853		U54NS115198	—	77,267
Massachusetts General Hospital	93.853		R01NS102574	—	(7,739)
Massachusetts General Hospital	93.853		R01NS128962	—	11,395
Massachusetts General Hospital	93.853		R01NS130119	—	286,971
Massachusetts General Hospital	93.853		R01NS130209	—	224,837
Massachusetts General Hospital	93.853		R01NS131347	—	49,871
Massachusetts General Hospital	93.853		R01NS096177	—	465
Massachusetts General Hospital	93.853		R61NS126776	—	75,999
Massachusetts General Hospital	93.853		RF1NS120947	—	(76,251)
Massachusetts General Hospital	93.853		RM1NS128741	—	402,554
Massachusetts General Hospital	93.853		U01NS114001	—	(861)
Massachusetts General Hospital	93.853		U01NS136021	—	6,177
Massachusetts General Hospital	93.853		U24NS107243	—	2,334
Mayo Clinic Rochester	93.853		U01NS080168	—	5,593
Mayo Clinic Rochester	93.853		U54NS115198	—	18,557
The University of Texas	93.853		R01NS119225	—	23,013
The University of Texas	93.853		UG3NS128397	—	26,162
University of California, Davis	93.853		R01NS073613	—	98,291

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University of California, Davis	93.853		R01NS118856	\$ —	33,056
University of Cincinnati	93.853		U01N06513	—	9,332
University of Cincinnati	93.853		U01NS110728	—	235
University of Cincinnati	93.853		U01NS110772	—	1,830
University of Cincinnati	93.853		U01NS120910	—	7,549
University of Cincinnati	93.853		U01NS095869	—	10,124
University of Cincinnati	93.853		U01NS099043	—	710
University of Cincinnati	93.853		UF1NS120871	—	300
University of Florida	93.853		U01NS119562	—	32,069
University of Michigan	93.853		U01NS099046	—	4,465
University of Pennsylvania	93.853		U01NS113198	—	73,908
University of Washington	93.853		R01NS138297	—	4,476
<i>Extramural Research Programs in the Neurosciences and Neurological Disorders Subtotal</i>				<u>3,110,859</u>	<u>12,183,348</u>
<i>National Institute of Neurological Disorders and Stroke Subtotal</i>				<u>3,117,904</u>	<u>12,288,947</u>
National Institute of Allergy and Infectious Diseases Trans-NIH Research Support	93.310		Direct	99,353	266,378
<i>Trans-NIH Research Support Subtotal</i>				<u>99,353</u>	<u>266,378</u>
Allergy and Infectious Diseases Research	93.855		Direct	5,196,463	17,886,818
Advanced Bioscience Laboratories	93.855		HHSN272201300002I	—	1,222
Benaroya Research Institute at Virginia Mason	93.855		UM1A1109565	—	21,017
Boston Children's Hospital	93.855		75N93020C00038	—	11,766
Boston Children's Hospital	93.855		HHSN272201800047C	—	(1,973)
Boston Children's Hospital	93.855		R01AI174938	—	147,980
Boston Children's Hospital	93.855		R01AI181618	—	56,344
Boston Children's Hospital	93.855		U19AI168643	—	10,847
Brigham and Women's Hospital	93.855		R01AI155131	—	4,531
Brigham and Women's Hospital	93.855		T32AI007387	—	119,338
Brigham and Women's Hospital	93.855		UM1AI069412	—	1,299,546
Center for Infectious Disease Research	93.855		75N93019C00070	—	423,881
CENTRO CIENTIFICO TECNOLÓGICO CONICET-CORDOBA (CUIT 30710178034)	93.855		R01AI176457	—	13,526
Dana-Farber Cancer Institute	93.855		P01AI181597	—	57,770
Dana-Farber Cancer Institute	93.855		R01AI165666	—	16,499
Dana-Farber Cancer Institute	93.855		T32AI007386	—	(292)
Duke University	93.855		P01AI158571	—	1,629,500
Duke University	93.855		U19AI142596	—	744,083
Duke University	93.855		UM1AI144371	—	1,731,149
Duke University	93.855		UM1AI169633	—	4,021,451
Emory University	93.855		U19AI110483	—	17,242
Food Allergy Science Initiative	93.855		P01AI179273	—	89,602
Fred Hutchinson Cancer Research Center	93.855		UM1AI068614	—	308,485
Harvard Medical School	93.855		P30AI060354	—	69,323
Harvard Pilgrim Health Care	93.855		R01AI174862	—	20,595
Harvard University	93.855		P30AI060354	—	117,333
Hillhurst Biopharmaceuticals, Inc.	93.855		R41AI164948	—	50,926
Icahn School of Medicine at Mount Sinai	93.855		75N93021C00014	—	126,538
ImmunogenX	93.855		U44AI134590	—	29,537
JBS Science, Inc.	93.855		R43AI167169	—	20,869

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JBS Science, Inc.	93.855		R43AI174349	\$ —	106,657
Johns Hopkins University	93.855		P01AI169615	—	685,227
Massachusetts General Hospital	93.855		T32AI007061	—	10,820
Massachusetts General Hospital	93.855		R21AI155152	—	(9,605)
Northeastern University	93.855		R21AI140212	—	3,901
Oregon Health & Science University	93.855		R01AI149672	—	54,651
The Henry M. Jackson Foundation	93.855		R01AI155983	—	215,841
The Rockefeller University	93.855		U01AI169385	—	124,680
University of Alabama at Birmingham	93.855		R01AI155052	—	5,227
University of California, San Francisco	93.855		R01AI098472	—	18,679
University of California, San Francisco	93.855		U01AI152087	—	21,045
University of Cape Town	93.855		R01AI160501	—	47,520
University of Toledo	93.855		R01AI184880	—	11,851
Vanderbilt University Medical Center	93.855		1 U19 AI181979-01	—	31,062
Vanderbilt University Medical Center	93.855		N01AI75N93019C00074	—	166,942
<i>Allergy and Infectious Diseases Research Subtotal</i>				<u>5,196,463</u>	<u>30,539,951</u>
<i>National Institute of Allergy and Infectious Diseases Subtotal</i>				<u>5,295,816</u>	<u>30,806,329</u>
National Institute of General Medical Sciences					
Biomedical Research and Research Training	93.859		Direct	159,720	1,503,391
Harvard Medical School	93.859		R35GM145248	—	124,563
Massachusetts General Hospital	93.859		R21GM148826	—	26,565
Massachusetts General Hospital	93.859		R21GM151565	—	83,928
Massachusetts General Hospital	93.859		T32GM007592	—	88,763
<i>National Institute of General Medical Sciences Subtotal</i>				<u>159,720</u>	<u>1,827,210</u>
National Institute of Child Health and Human Development					
Child Health and Human Development Extramural Research	93.865		Direct	387,830	1,325,722
Boston Children's Hospital	93.865		R01HD094794	—	8,834
Brigham and Women's Hospital	93.865		DP2HD112176	—	(11,555)
Brigham and Women's Hospital	93.865		K12HD051959	—	41,176
Children's Research Institute	93.865		R01HD089289	—	3,723
Harvard T.H. Chan School School of Public Health	93.865		P01HD107670	—	62,729
Johns Hopkins University	93.865		R01HD041702	—	200,634
Medical University of South Carolina	93.865		P2CHD086844	—	1,260
Oregon Health & Science University	93.865		R01HD105655	—	13,469
Regents of the University of Minnesota	93.865		R01HD080444	—	26,979
Regents of the University of Minnesota	93.865		R01HD109830	—	6,048
Shirley Ryan AbilityLab	93.865		P2CHD101899	—	14,218
Stanford University	93.865		P2CHD101913	—	24,814
Stanford University	93.865		R01HD108794	—	17,376
The Curators of the Uni of MO	93.865		K12HD000849	—	18,225
University Health Network 3-464	93.865		R01HD104553	—	15,053
University of Massachusetts	93.865		R15HD110945	—	18,488
Yale University	93.865		R01HD106359	—	140,744
<i>National Institute of Child Health and Human Development Subtotal</i>				<u>387,830</u>	<u>1,927,937</u>

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National Institute on Aging					
Aging Research	93.866		Direct	\$ 3,196,858	14,507,201
Biosensics, LLC	93.866		R44AG065014	—	(45,932)
Biosensics, LLC	93.866		R44AG081031	—	102,812
Biosensics, LLC	93.866		R44AG084327	—	102,639
Boston University	93.866		RF1AG063507	—	(934)
Boston University	93.866		U19AG073172	—	438,960
Boston University	93.866		U19AG076471	—	55,029
Brigham and Women's Hospital	93.866		P30AG031679	—	1,822
Brigham and Women's Hospital	93.866		R01AG071074	—	16,931
Brigham and Women's Hospital	93.866		R01AG074932	—	41,144
Brigham and Women's Hospital	93.866		R21AG075928	—	10,972
Brigham and Women's Hospital	93.866		RF1AG064312	—	46,971
Brigham and Women's Hospital	93.866		U54AG062322	—	84,547
Buck Institute	93.866		R56AG082819	—	95,752
California Pacific Med Ctr Research Inst	93.866		R01AG066671	—	222,444
CND Life Sciences, Inc.	93.866		R44AG076072	—	132,896
Duke University	93.866		R01AG073598	—	71,327
George Washington University	93.866		U19-AG078558	—	185,482
Harvard T.H. Chan School of Public Health	93.866		R01AG081151	—	30,471
Hebrew Rehabilitation Center	93.866		K23AG066929	—	183,310
Hebrew Rehabilitation Center	93.866		P01AG031720	—	190,718
Hebrew Rehabilitation Center	93.866		R01AG044518	—	125,379
Hebrew Rehabilitation Center	93.866		R01AG065229	—	74,666
Hebrew Rehabilitation Center	93.866		R33AG071744	8,022	126,359
Hebrew Rehabilitation Center	93.866		T32AG023480	—	149,998
Johns Hopkins University	93.866		R01AG066741	—	33,612
Johns Hopkins University	93.866		R01AG066768	5,605	633,736
Johns Hopkins University	93.866		R35AG072310	—	48,406
Massachusetts General Hospital	93.866		R01AG060981	—	146,558
Massachusetts General Hospital	93.866		R01AG062547	—	237,503
Massachusetts General Hospital	93.866		R01AG062667	—	71,195
Massachusetts General Hospital	93.866		R01AG073410	—	321
Massachusetts General Hospital	93.866		R01AG083777	—	29,466
Massachusetts General Hospital	93.866		R21AG081763	—	22,201
Northeastern University	93.866		R33AG057979	—	395
Northern California Institute for Research and Education	93.866		R01AG027002	—	1,233
Northern California Institute for Research and Education	93.866		R01AG079982	—	69,072
Northern California Institute for Research and Education	93.866		R24AG064025	—	114,147
Northwestern University	93.866		R01AG078261	—	23,647
Parkinson's Foundation	93.866		R01AG059417	—	26,694
Pennsylvania State University	93.866		R01AG030618	—	125,564
Rush University Medical Center	93.866		R01AG062711	—	4,902
Rush University Medical Center	93.866		P30AG072975	—	11,289
Rush University Medical Center	93.866		R01AG075728	—	145,202
Rutgers State University of New Jersey	93.866		R01AG063937	—	18,756
The University of North Carolina at CH	93.866		U01AG082042	—	28,182
The University of North Carolina at CH	93.866		R21AG080331	—	28,047
The University of Texas	93.866		RF1AG063507	—	894,490

**BETH ISRAEL LAHEY HEALTH, INC.
AND AFFILIATES (BIDMC SEFA)**

Supplementary Schedule of Expenditures of Federal Awards

Year ended September 30, 2024

Federal grantor/pass-through grantor/program title	Assistance listing number	COVID	Direct or pass-through identifying number	Passed through to subrecipients	Total Federal Expenditures
University of California, Davis	93.866		R01AG078283	\$ —	23,111
University of California, San Diego	93.866		R01AG075758	—	155,681
University of California, San Diego	93.866		RF1AG079149	—	3,267,207
University of California, San Francisco	93.866		R21AG085495	—	26,000
University of Colorado Denver	93.866		R01AG077956	—	91,073
University of Massachusetts	93.866		R01AG055948	—	6,911
University of Massachusetts Lowell	93.866		R01AG055948	—	35,739
University of Pennsylvania	93.866		P30AG073104	—	74,129
University of Pittsburgh	93.866		K7AG074878	—	56,504
Washington University	93.866		R01AG074012	—	28,734
Washington University	93.866		RF1AG071706	—	(39,056)
Washington University	93.866		RF1AG083753	—	29,116
<i>National Institute on Aging Subtotal</i>				<u>3,210,485</u>	<u>23,420,731</u>
National Eye Institute					
Vision Research	93.867		Direct	242,288	1,407,865
Boston Children's Hospital	93.867		R01EY013613	—	61,122
JAEB Center for Health Research Foundation, Inc	93.867		UG1EY014231	—	758,646
New York University	93.867		U10EY026869	—	5,830
Schepens Eye Research Institute	93.867		R21EY030631	—	—
Physical Sciences	93.867		R43EY029927	—	—
<i>National Eye Institute Subtotal</i>				<u>242,288</u>	<u>2,233,463</u>
National Library of Medicine					
Medical Library Assistance					
Harvard Medical School	93.879		T15LM007092	—	(5,000)
<i>National Library of Medicine Subtotal</i>				<u>—</u>	<u>(5,000)</u>
<i>National Institutes of Health Subtotal</i>				<u>33,139,803</u>	<u>208,898,338</u>
<i>Department of Health and Human Services Total</i>				<u>33,329,076</u>	<u>211,934,109</u>
Department of State:					
Office of Foreign Assistance					
USAID Foreign Assistance for Programs Overseas					
Brigham and Women's Hospital	98.001		AID-440-A-16-00002	—	927,297
<i>Department of State Total</i>				<u>—</u>	<u>927,297</u>
Total Expenditures of Research and Development Federal Awards				<u>34,265,585</u>	<u>223,524,879</u>
Other Federal Awards:					
Department of Justice:					
Office for Victims of Crime					
Services for Trafficking Victims	16.320		Direct	47,967	105,420
Mass Office For Victim Assistance	16.320		VOCA2023BIDM000000000	—	(6,093)
<i>Department of Justice Total</i>				<u>47,967</u>	<u>99,327</u>

**BETH ISRAEL LAHEY HEALTH, INC.
AND AFFILIATES (BIDMC SEFA)**

Supplementary Schedule of Expenditures of Federal Awards

Year ended September 30, 2024

Federal grantor/pass-through grantor/program title	Assistance listing number	COVID	Direct or pass-through identifying number	Passed through to subrecipients	Total Federal Expenditures
Department of Health and Human Services:					
Centers for Disease Control					
Centers for Disease Control - Maternal Mortality Prevention Team	93.UNK		Direct	\$ —	1,296
<i>Centers for Disease Control - Maternal Mortality Prevention Team Subtotal</i>				—	1,296
Global AIDS	93.067		Direct	—	1,132,752
<i>Global AIDS Subtotal</i>				—	1,132,752
<i>Centers for Disease Control Subtotal</i>				—	1,134,048
Health Resources and Services Administration					
HIV-Related Training and Technical Assistance					
Community Research Initiative	93.145		5 U1OHA29294-09	—	41,564
<i>HIV-Related Training and Technical Assistance Subtotal</i>				—	41,564
Hansen's Disease National Ambulatory Care Program	93.215		Direct	—	68,332
<i>Hansen's Disease National Ambulatory Care Program Subtotal</i>				—	68,332
The National Cardiovascular Health Program					
JSI Research and Training Institute, Inc.	93.426		37824.2025.0001,37824.2025.0002	—	27,500
<i>The National Cardiovascular Health Program Subtotal</i>				—	27,500
Provider Relief Fund and American Rescue Plan	93.498	C	Direct	—	48,050
<i>Provider Relief Fund and American Rescue Plan Subtotal</i>				—	48,050
Healthy Start Initiative					
Boston Public Health Commission	93.926		H49MC00128-22-00	—	(2,347)
Boston Public Health Commission	93.926		H49MC00128-23-00	—	49,445
Boston Public Health Commission	93.926		H49MC00128-24-00	—	34,067
<i>Health Resources and Services Administration Subtotal</i>				—	81,165
HIV Emergency Relief Project Grants					
Boston Public Health Commission	93.914		H89HA00011	—	126,950
<i>HIV Emergency Relief Project Grants Subtotal</i>				—	126,950
Outpatient Early Intervention Services with Respect to HIV Disease	93.918		Direct	—	197,211
<i>Outpatient Early Intervention Services with Respect to HIV Disease Subtotal</i>				—	197,211
<i>Health Resources and Services Administration Subtotal</i>				—	590,772
Indian Health Service					
Vision Network Teleophthalmology Program					
Estenda	93.U01		HHSSI236201600002C	—	229,749
<i>Indian Health Service Subtotal</i>				—	229,749

**BETH ISRAEL LAHEY HEALTH, INC.
AND AFFILIATES (BIDMC SEFA)**

Supplementary Schedule of Expenditures of Federal Awards

Year ended September 30, 2024

Federal grantor/pass-through grantor/program title	Assistance listing number	COVID	Direct or pass-through identifying number	Passed through to subrecipients	Total Federal Expenditures
Substance Abuse and Mental Health Services Administration					
Opioid STR					
Massachusetts Department of Public Health	93.788		INTF2306M03193328008	\$ —	227,183
Massachusetts Department of Public Health	93.788		INTF2306M03193328034	—	227,183
<i>Opioid STR Subtotal</i>				—	<u>454,367</u>
Block Grants for Prevention and Treatment of Substance Abuse					
Massachusetts Department of Public Health	93.959		INTF2301MM3160524176	—	2,055
Massachusetts Department of Public Health	93.959		INTF2303MM3182026080	—	116,571
Massachusetts Department of Public Health	93.959		INTF2306M03193328008	—	20,194
Massachusetts Department of Public Health	93.959		INTF2306M03193328034	—	20,194
Massachusetts Department of Public Health	93.959		INTF2318MM3160424036	—	127,440
Massachusetts Department of Public Health	93.959		INTF2324MM3210930004	—	12,222
<i>Block Grants for Prevention and Treatment of Substance Abuse Subtotal</i>				—	<u>298,676</u>
<i>Substance Abuse and Mental Health Services Administration Subtotal</i>				—	<u>753,043</u>
<i>Department of Health and Human Services Total</i>				—	<u>2,707,612</u>
Department of Homeland Security:					
Disaster Grants - Public Assistance					
Massachusetts Emergency Management Agency	97.036	C	4496DR-MA	—	20,275,139
<i>Department of Homeland Security Total</i>				—	<u>20,275,139</u>
Department of Treasury:					
Coronavirus State And Local Fiscal Recovery Funds					
Vinfen Corporation	21.027	C	Vinfen Corporation	—	248,128
<i>Department of Treasury Total</i>				—	<u>248,128</u>
Total expenditures of other federal awards				47,967	23,330,206
Total expenditures of all federal awards				<u>\$ 34,313,553</u>	<u>246,855,084</u>

See accompanying notes to schedule of expenditures of federal awards.

**BETH ISRAEL LAHEY HEALTH, INC.
AND AFFILIATES**

Notes to Schedule of Expenditures of Federal Awards

September 30, 2024

(1) Definition of Reporting Entity

The schedule of expenditures of federal awards (the Schedule) presents the activity of all federal awards of Beth Israel Lahey Health, Inc. and Affiliates (BILH). All federal awards received directly from federal agencies, as well as federal awards passed through other agencies, are included on the Schedule.

(2) Basis of Presentation

The accompanying Schedule is presented using the cash basis of accounting. The information in the Schedule is presented in accordance with the requirements of Title 2 U.S. Code of Federal Regulation Part 200, *Uniform Administrative Requirements, Cost Principles, and Audit Requirements for Federal Awards (Uniform Guidance)*. Negative amounts shown on the Schedule represent adjustments or credits made in the normal course of business to amounts reported as expenditures in prior years.

(3) Indirect Costs

Indirect costs are charged to federal grants and contracts at a federally approved predetermined rate. Indirect costs recovered are included in reported federal expenditures. BILH has not elected to use the 10% de minimis indirect cost rate allowed under the Uniform Guidance.



KPMG LLP
Two Financial Center
60 South Street
Boston, MA 02111

**Independent Auditors' Report on Internal Control Over Financial Reporting and on
Compliance and Other Matters Based on an Audit of Financial Statements
Performed in Accordance With *Government Auditing Standards***

Board of Trustees
Beth Israel Lahey Health, Inc. and Affiliates:

We have audited, in accordance with the auditing standards generally accepted in the United States of America and the standards applicable to financial audits contained in *Government Auditing Standards* issued by the Comptroller General of the United States, the consolidated financial statements of Beth Israel Lahey Health, Inc. and Affiliates (BILH), which comprise BILH's consolidated balance sheet as of September 30, 2024, and the related consolidated statements of operations, changes in net assets, and cash flows for the year then ended, and the related notes to the consolidated financial statements, and have issued our report thereon dated December 17, 2024.

Report on Internal Control Over Financial Reporting

In planning and performing our audit of the consolidated financial statements, we considered BILH's internal control over financial reporting (internal control) as a basis for designing audit procedures that are appropriate in the circumstances for the purpose of expressing our opinion on the consolidated financial statements, but not for the purpose of expressing an opinion on the effectiveness of BILH's internal control. Accordingly, we do not express an opinion on the effectiveness of BILH's internal control.

A deficiency in internal control exists when the design or operation of a control does not allow management or employees, in the normal course of performing their assigned functions, to prevent, or detect and correct, misstatements on a timely basis. A material weakness is a deficiency, or a combination of deficiencies, in internal control, such that there is a reasonable possibility that a material misstatement of the entity's financial statements will not be prevented, or detected and corrected, on a timely basis. A significant deficiency is a deficiency, or a combination of deficiencies, in internal control that is less severe than a material weakness, yet important enough to merit attention by those charged with governance.

Our consideration of internal control was for the limited purpose described in the first paragraph of this section and was not designed to identify all deficiencies in internal control that might be material weaknesses or significant deficiencies. Given these limitations, during our audit we did not identify any deficiencies in internal control that we consider to be material weaknesses. However, material weaknesses or significant deficiencies may exist that were not identified.

Report on Compliance and Other Matters

As part of obtaining reasonable assurance about whether BILH's consolidated financial statements are free from material misstatement, we performed tests of its compliance with certain provisions of laws, regulations, contracts, and grant agreements, noncompliance with which could have a direct and material effect on the financial statements. However, providing an opinion on compliance with those provisions was not an objective of our audit, and accordingly, we do not express such an opinion. The results of our tests disclosed no instances of noncompliance or other matters that are required to be reported under *Government Auditing Standards*.



Purpose of This Report

The purpose of this report is solely to describe the scope of our testing of internal control and compliance and the results of that testing, and not to provide an opinion on the effectiveness of the entity's internal control or on compliance. This report is an integral part of an audit performed in accordance with *Government Auditing Standards* in considering the entity's internal control and compliance. Accordingly, this communication is not suitable for any other purpose.

KPMG LLP

Boston, Massachusetts
December 17, 2024



KPMG LLP
Two Financial Center
60 South Street
Boston, MA 02111

**Independent Auditors' Report on Compliance for The Major Federal Program;
Report on Internal Control Over Compliance; and Report on Schedule of
Expenditures of Federal Awards Required by the Uniform Guidance**

Board of Trustees
Beth Israel Lahey Health, Inc. and Affiliates:

Report on Compliance for The Major Federal Program

Opinion on The Major Federal Program

We have audited Beth Israel Lahey Health, Inc. and Affiliates (BILH) compliance with the types of compliance requirements identified as subject to audit in the *OMB Compliance Supplement* that could have a direct and material effect on BILH's major federal program for the year ended September 30, 2024. BILH's major federal program is identified in the summary of auditors' results section of the accompanying schedule of findings and questioned costs.

In our opinion, BILH complied, in all material respects, with the compliance requirements referred to above that could have a direct and material effect on its major federal program for the year ended September 30, 2024.

Basis for Opinion on The Major Federal Program

We conducted our audit of compliance in accordance with auditing standards generally accepted in the United States of America (GAAS); the standards applicable to financial audits contained in *Government Auditing Standards* issued by the Comptroller General of the United States; and the audit requirements of Title 2 U.S. Code of Federal Regulations Part 200, *Uniform Administrative Requirements, Cost Principles, and Audit Requirements for Federal Awards* (Uniform Guidance). Our responsibilities under those standards and the Uniform Guidance are further described in the Auditors' Responsibilities for the Audit of Compliance section of our report.

We are required to be independent of BILH and to meet our other ethical responsibilities, in accordance with relevant ethical requirements relating to our audit. We believe that the audit evidence we have obtained is sufficient and appropriate to provide a basis for our opinion on compliance for is major federal program. Our audit does not provide a legal determination of BILH's compliance with the compliance requirements referred to above.

Responsibilities of Management for Compliance

Management is responsible for compliance with the requirements referred to above and for the design, implementation, and maintenance of effective internal control over compliance with the requirements of laws, statutes, regulations, rules and provisions of contracts or grant agreements applicable to BILH's federal programs.

Auditors' Responsibilities for the Audit of Compliance

Our objectives are to obtain reasonable assurance about whether material noncompliance with the compliance requirements referred to above occurred, whether due to fraud or error, and express an opinion on BILH's compliance based on our audit. Reasonable assurance is a high level of assurance but is not absolute assurance and therefore is not a guarantee that an audit conducted in accordance with GAAS, *Government Auditing Standards*, and the Uniform Guidance will always detect material noncompliance when it exists. The risk of not detecting material noncompliance resulting from fraud is higher than for that resulting from error, as



fraud may involve collusion, forgery, intentional omissions, misrepresentations, or the override of internal control. Noncompliance with the compliance requirements referred to above is considered material if there is a substantial likelihood that, individually or in the aggregate, it would influence the judgment made by a reasonable user of the report on compliance about BILH's compliance with the requirements of the major federal program as a whole.

In performing an audit in accordance with GAAS, *Government Auditing Standards*, and the Uniform Guidance, we:

- Exercise professional judgment and maintain professional skepticism throughout the audit.
- Identify and assess the risks of material noncompliance, whether due to fraud or error, and design and perform audit procedures responsive to those risks. Such procedures include examining, on a test basis, evidence regarding BILH's compliance with the compliance requirements referred to above and performing such other procedures as we considered necessary in the circumstances.
- Obtain an understanding of BILH's internal control over compliance relevant to the audit in order to design audit procedures that are appropriate in the circumstances and to test and report on internal control over compliance in accordance with the Uniform Guidance, but not for the purpose of expressing an opinion on the effectiveness of BILH's internal control over compliance. Accordingly, no such opinion is expressed.

We are required to communicate with those charged with governance regarding, among other matters, the planned scope and timing of the audit and any significant deficiencies and material weaknesses in internal control over compliance that we identified during the audit.

Other Matters

The results of our auditing procedures disclosed an instance of noncompliance which is required to be reported in accordance with the Uniform Guidance and which is described in the accompanying schedule of findings and questioned costs as item 2024-001. Our opinion on the major federal program is not modified with respect to this matter.

Government Auditing Standards requires the auditor to perform limited procedures on BILH's responses to the noncompliance findings identified in our compliance audit described in the accompanying schedule of findings and questioned costs. BILH is also responsible for preparing a corrective action plan to address each audit finding included in our auditors' report. BILH's responses and corrective action plan were not subjected to the other auditing procedures applied in the audit of compliance and, accordingly, we express no opinion on the responses or the corrective action plan.

Report on Internal Control Over Compliance

Our consideration of internal control over compliance was for the limited purpose described in the Auditors' Responsibilities for the Audit of Compliance section above and was not designed to identify all deficiencies in internal control over compliance that might be material weaknesses or significant deficiencies in internal control over compliance and therefore, material weaknesses or significant deficiencies may exist that were not identified. We did not identify any deficiencies in internal control that we considered to be material weaknesses. However, as discussed below, we did identify a deficiency in internal control over compliance that we consider to be a significant deficiency.

A deficiency in internal control over compliance exists when the design or operation of a control over compliance does not allow management or employees, in the normal course of performing their assigned functions, to prevent, or detect and correct, noncompliance with a type of compliance requirement of a federal program on a timely basis. A material weakness in internal control over compliance is a deficiency, or a combination of deficiencies, in internal control over compliance, such that there is a reasonable possibility that



material noncompliance with a type of compliance requirement of a federal program will not be prevented, or detected and corrected, on a timely basis. A significant deficiency in internal control over compliance is a deficiency, or a combination of deficiencies, in internal control over compliance with a type of compliance requirement of a federal program that is less severe than a material weakness in internal control over compliance, yet important enough to merit attention by those charged with governance. We consider the deficiency in internal control over compliance described in the accompanying schedule of findings and questioned costs as item 2024-001 to be a significant deficiency.

Our audit was not designed for the purpose of expressing an opinion on the effectiveness of internal control over compliance. Accordingly, no such opinion is expressed.

Government Auditing Standards requires the auditor to perform limited procedures on BILH's responses to the internal control over compliance findings identified in our audit described in the accompanying schedule of findings and questioned costs. BILH is also responsible for preparing a corrective action plan to address each audit finding included in our auditors' report. BILH's responses and corrective action plan were not subjected to the other auditing procedures applied in the audit of compliance and, accordingly, we express no opinion on the responses or the corrective action plan.

The purpose of this report on internal control over compliance is solely to describe the scope of our testing of internal control over compliance and the results of that testing based on the requirements of the Uniform Guidance. Accordingly, this report is not suitable for any other purpose.

Report on Schedule of Expenditures of Federal Awards Required by the Uniform Guidance

We have audited the consolidated financial statements of Beth Israel Lahey Health, Inc. and Affiliates as of and for the year ended September 30, 2024, and have issued our report thereon dated December 17, 2024, which contained an unmodified opinion on those consolidated financial statements. Our audit was conducted for the purpose of forming an opinion on the consolidated financial statements as a whole. The accompanying schedule of expenditures of federal awards is presented for purposes of additional analysis as required by the Uniform Guidance and is not a required part of the consolidated financial statements. Such information is the responsibility of management and was derived from and relates directly to the underlying accounting and other records used to prepare the consolidated financial statements. The information has been subjected to the auditing procedures applied in the audit of the consolidated financial statements and certain additional procedures, including comparing and reconciling such information directly to the underlying accounting and other records used to prepare the consolidated financial statements or to the consolidated financial statements themselves, and other additional procedures in accordance with GAAS. In our opinion, the schedule of expenditures of federal awards is fairly stated in all material respects in relation to the consolidated financial statements as a whole.

KPMG LLP

Boston, Massachusetts
June 27, 2025

**BETH ISRAEL LAHEY HEALTH, INC.
AND AFFILIATES**

Schedule of Findings and Questioned Costs

Year ended September 30, 2024

(1) Summary of Auditors' Results

Consolidated Financial Statements

- a) Type of report issued on whether the financial statements were prepared in accordance with U.S. generally accepted accounting principles: **Unmodified**
- b) Internal control deficiencies over financial reporting disclosed by the audit of the financial statements:
 - Material weaknesses: **No**
 - Significant deficiencies: **None reported**
- c) Noncompliance material to the financial statements: **No**

Federal Awards

- d) Internal control deficiencies over major program disclosed by the audit:
 - Material weaknesses: **No**
 - Significant deficiencies: **2024-001**
- e) Type of report issued on compliance for the major program: **Unmodified**
- f) Audit findings that are required to be reported in accordance with 2 CFR 200.516(a): **Yes**
- g) Major program:

Research and Development Cluster – Various
- h) Dollar threshold used to distinguish between Type A and Type B programs: **\$3,000,000**
- i) Auditee qualified as a low-risk auditee: **No**

(2) Findings Relating to Financial Statements Reported in Accordance with *Government Auditing Standards*

None.

**BETH ISRAEL LAHEY HEALTH, INC.
AND AFFILIATES**

Schedule of Findings and Questioned Costs

Year ended September 30, 2024

(3) Findings and Questioned Costs Relating to Federal Awards

Finding 2024-001 – Subrecipient Monitoring

Federal Agency: United States Department of Health and Human Services

Program: Research and Development Cluster

Assistance Listing Number: 93.837 and 93.847

Criteria:

Criteria: 2 CFR 200.332(b) requires pass-through entities to evaluate each subrecipient's risk of non-compliance with Federal statutes, regulations, and the terms and conditions of the subaward for purposes of determining the appropriate subrecipient monitoring described in paragraphs (d) and (e) of 200.332. Further, 200.332(d) requires pass-through entities to monitor the activities of the subrecipient as necessary to ensure that the subaward is used for authorized purposes, in compliance with Federal statutes, regulations, and the terms and conditions of the subaward; and that subaward performance goals are achieved.

Condition:

Annually, BILH works with over 200 subrecipients in conducting its research activities. While Joslin Diabetes Center (the Center) has a subrecipient pre-award risk assessment process in place, for 5 of 9 subrecipients selected for testwork, we were unable to verify that the pre-award risk assessment procedures were fully performed. Additionally, while the Center's risk assessment procedures correspond with a risk-based monitoring plan, for 8 of 9 subrecipients selected for testwork, we were unable to verify that the required monitoring activities were fully completed.

We deemed this to be a significant deficiency in internal controls.

Cause:

As a result of turnover during the fiscal year within Research Finance there was a lack of effective operation of internal controls over subrecipient risk assessment and monitoring activities.

Possible Asserted Effect:

Without the proper operation of an effectively designed system of internal controls, the internal control system cannot be relied upon to effectively prevent or detect non-compliance.

Questioned Costs:

None.

Statistical Sampling:

The sample was not intended to be, and was not, a statistically valid sample.

Identification of Whether the Audit Finding was a Repeat Finding:

This is a repeat finding. (Prior year finding 2023-002)

**BETH ISRAEL LAHEY HEALTH, INC.
AND AFFILIATES**

Schedule of Findings and Questioned Costs

Year ended September 30, 2024

Recommendation:

We recommend that the Center evaluate its current Subrecipient Monitoring and Management Policy to ensure that the policy reflects each of the risk assessment and monitoring compliance requirements of the Uniform Guidance. Additionally, the results of the Center's risk assessment and monitoring activities should be formally documented within the tracking log which should then be subject to formal review and approval by an appropriate individual within Research Finance.

Views of Responsible Officials:

Management acknowledges although tracking logs had been maintained, due to personnel turnover during the year, not all had been reviewed. Management has since created a shared email address to ensure communication and information sharing with all necessary parties. Management is reviewing its current policy for opportunities to strengthen internal controls and current procedures while utilizing available resources.

Management Response and Corrective Action Plan

Finding 2024-001 – Subrecipient Monitoring

Federal Agency: United States Department of Health and Human Services

Program Name: Research and Development (R&D)

Assistance Listing Number: Various

Responsible Individual: Roy Bourne, Director, Research Finance and Operations

Contact Information: rbourne2@joslin.harvard.edu; 617-309-5741

Joslin Diabetes Center's (Center) subrecipient monitoring process did not clearly indicate risk assessment procedures or the required monitoring activities in certain audited instances. While the Center has a Subrecipient Monitoring and Management policy, review suggests that a thorough evaluation of this plan, formal documentation, and secondary oversight will improve internal control. Management agrees with the recommendation and will evaluate the subrecipient monitoring process according to 2 CFR 200.332 and update established policy where applicable.

Corrective Action Plan:

- Management completed the review of the Subrecipient Monitoring and Management policy for relevant updates and improvements to internal control as of May 2025
- Results of risk assessment procedures and subrecipient monitoring will be formally documented within the tracking log
- Log entries were updated to reflect a reviewers note documenting material and date of review as of May 2025
- Director of Research Finance and Operations will review log semi-annually for secondary oversight

Expected Completion Date: June 30, 2025

Status of Completion: Partially Corrected

BETH ISRAEL LAHEY HEALTH, INC. AND AFFILIATES

Summary of Schedule of Status of Prior Year Findings and Questioned Costs for year ended September 30, 2023

Finding 2023-001 – Equipment

Federal Agency: United States Department of Health and Human Services

Program Name: Research and Development Cluster

Assistance Listing Number: Various

Criteria:

According to 2 CFR 200.313(d)(1), property records must be maintained that include a description of the property, a serial number or other identification number, the source of funding for the property, who holds title, the acquisition date, and cost of the property, percentage of Federal participation in the project costs for the Federal award under which the property was acquired, the location, use and condition of the property, and any ultimate disposition data including the date of disposal and sale price of the property.

Per 2 CFR 200.313(d)(3) and (4), a control system must be developed to ensure adequate safeguards to prevent loss, damage, or theft of property and adequate maintenance procedures must be developed to keep the property in good condition.

Additionally, 2 CFR section 200.313(d)(2) requires a physical inventory of Federally acquired property must be taken and the results reconciled with the property records at least once every two years.

In addition, 2 CFR 200.303 requires nonfederal entities to, among other things, establish and maintain effective internal control over the Federal award that provides reasonable assurance that the non-Federal entity is managing the Federal award in compliance with Federal statutes, regulations, and the terms and conditions of the Federal award. Effective internal controls should include procedures to ensure that accurate property records are maintained and equipment items are property tagged.

Condition:

Beth Israel Deaconess Medical Center (BIDMC), one of Beth Israel Lahey Health, Inc.'s affiliates, did not consistently follow its property management policies and procedures related to maintaining accurate property management records for equipment purchased with Federal R&D Cluster program awards.

BIDMC conducts research at multiple locations throughout its campus where equipment purchased with Federal awards is utilized and maintained. As of September 30, 2023, BIDMC maintained Federally acquired property of approximately \$65.5 million. BIDMC identifies all equipment in its property management records with individually assigned asset numbers and each individual asset record includes the specific location of the asset, the Federal award general ledger account number which funded the purchase of the asset, and other required information. An asset tag with the assigned asset number is affixed to each asset in accordance with BIDMC policy. BIDMC is required to safeguard equipment purchased with Federal awards, maintain current property records, and to perform a physical inventory of equipment purchased with Federal awards on a biennial basis.

During our physical inspection of 40 pieces of equipment (with a cost basis of \$4,562,717) purchased with Federal funds, we identified 7 items (with cost basis totaling \$116,831) selected for physical observation which had not been tagged. Overall, BIDMC's inventory listing included 34 pieces of equipment (with a cost basis of \$1,500,686) acquired with Federal funds that were not tagged. Additionally, we noted that BIDMC did not conduct a physical inventory within the biennial period required.

We deemed this to be a material weakness in internal controls.

Cause:

In discussing these conditions with BILH management, they stated the implementation of a new asset tagging system resulted in delayed tagging of purchased equipment as well as the physical inventory of Federally acquired property.

Possible Asserted Effect:

Failure to maintain accurate property records may prohibit BIDMC from properly safeguarding and maintaining equipment.

Questioned Costs:

None.

Statistical Sampling:

The sample was not intended to be, and was not, a statistically valid sample.

Identification of Whether the Audit Finding was a Repeat Finding:

This is not a repeat finding.

Recommendation:

We recommend BIDMC review its procedures for updating property records to ensure they accurately reflect equipment information. Additionally, management should arrange for a physical inspection and

reconciliation of Federally acquired property to books and records.

Status: Fully Corrected

Finding 2023-002 – Subrecipient Monitoring

Federal Agency: United States Department of Health and Human Services

Program: Research and Development Cluster

Assistance Listing Number: 93.837 and 93.847

Criteria:

Criteria: 2 CFR 200.332(b) requires pass-through entities to evaluate each subrecipient's risk of noncompliance

with Federal statutes, regulations, and the terms and conditions of the subaward for purposes of determining the appropriate subrecipient monitoring described in paragraphs (d) and (e) of 200.332.

Further, 200.332(d) requires pass-through entities to monitor the activities of the subrecipient as necessary to ensure that the subaward is used for authorized purposes, in compliance with Federal statutes, regulations, and the terms and conditions of the subaward; and that subaward performance goals are achieved.

Condition:

While Joslin Diabetes Center (the Center) has a subrecipient pre-award risk assessment process in place, for 19 of 25 subrecipients, we were unable to verify that the pre-award risk assessment procedures were fully performed. Additionally, while the Center's risk assessment procedures correspond with a risk-based monitoring plan, for 7 of 25 subrecipients, we were unable to verify that the required monitoring activities were fully completed.

We deemed this to be a significant deficiency in internal controls.

Cause:

As a result of turnover during the fiscal year within Research Finance there was a lack of effective operation of internal controls over subrecipient risk assessment and monitoring activities.

Possible Asserted Effect:

Without the proper operation of an effectively designed system of internal controls, the internal control system cannot be relied upon to effectively prevent or detect non-compliance.

Questioned Costs:

None.

Statistical Sampling:

The sample was not intended to be, and was not, a statistically valid sample.

Identification of Whether the Audit Finding was a Repeat Finding:

This is not a repeat finding.

Recommendation:

We recommend that the Center evaluate its current Subrecipient Monitoring and Management Policy to ensure that the policy reflects each of the risk assessment and monitoring compliance requirements of the Uniform Guidance. Additionally, the results of the Center's risk assessment and monitoring activities should be formally documented within the tracking log which should then be subject to formal review and approval by an appropriate individual within Research Finance.

Status: Partially Corrected

Finding 2023-003 – Reporting

Program: Provider Relief Fund and American Rescue Plan

Federal Agency: Health Resources and Services Administration

Assistance Listing Number: 93.498

Criteria:

Title 2 U.S. Code of Federal Regulations Part 200, Uniform Administrative Requirements, Cost Principles, and Audit Requirements for Federal Awards (Uniform Guidance) Subpart F – Audit Requirements §200.510 requires the auditee to prepare a schedule of expenditures of Federal awards for the period covered by the auditee's financial statements which must include the total Federal awards expended as determined in accordance with §200.502. The Uniform Guidance requires the auditor to determine whether the auditee's schedule of expenditures of federal awards is fairly stated, in all material respects, in relation to the audited entity's financial statements as a whole. In addition, the Uniform Guidance places the responsibility for identifying major programs on the auditor and the schedule of expenditures of federal awards serves as the primary basis for the auditor's major program determination. Therefore, appropriate major program determination by the auditor is dependent on the accuracy and completeness of the information in the schedule of expenditures of federal awards.

Condition:

In preparation of the schedule of expenditures for federal awards (SEFA) management did not record the Provider Relief Fund and American Rescue Plan expenditures on the SEFA which resulted in the SEFA

being understated by \$19,062,787.

We deemed this to be a material weakness in internal controls.

Cause:

BILH did not have an adequately designed internal control to identify the error in federal awards expended as such, the amount of the errors could have been greater and not detected by BILH's control environment.

Possible Asserted Effect:

Potential misstatement to the schedule of expenditures of federal awards.

Questioned Costs:

None noted

Statistical Sampling:

The sample was not intended to be, and was not, a statistically valid sample.

Identification of Whether the Audit Finding was a Repeat Finding:

This is a repeat finding.

Recommendation:

We recommend that BILH implement a more thorough and detailed process for preparing and reviewing the SEFA to identify adjustments that could result in a misstatement of the SEFA. This should include a formalized and detailed review of the SEFA and a reconciliation to the general ledger and financial statements by someone other than the preparer.

Status: Fully Corrected

Summary Schedule of Status of Prior Year Findings and Questioned Costs for year ended September 30, 2022

Finding 2022-001

Federal Agency: United States Department of Health and Human Services
Program Name: Provider Relief Fund (PRF)
Assistance Listing Number: 93.498

Criteria:

PRF recipients that received one or more payments exceeding \$10,000 in the aggregate during a Payment Received Period are required to report on several required data elements as part of the post-payment reporting process. Reporting must be completed and submitted to the Health Resources and Service Administration (HRSA) the reporting dates specified by HRSA. Additionally, Title 45 U.S. Code of Federal Regulations Part 75 (45 CFR 75), *Uniform Administrative Requirements, Cost Principles, and Audit Requirements* for HHS Awards, section 03(a) states the non-Federal entity must establish and maintain effective internal control over the Federal award that provides reasonable assurance that the non-Federal entity is managing the Federal award in compliance with Federal statutes, regulations, and the terms and conditions of the Federal award. These internal controls should be in compliance with guidance in "Standards for Internal Control in the Federal Government" issued by the Comptroller General of the United States or the "Internal Control Integrated Framework", issued by the Committee of Sponsoring Organizations of the Treadway Commission (COSO).

Condition:

During our audit procedures for Beth Israel Lahey Health's PRF Reporting Periods 2 and 3, we identified an error in the amount reported on the Lost Revenue in Excess of Total Distributions line item for the period of January 2020 to June 2021 for Beth Israel Deaconess Medical Center (BIDMC). Management incorrectly utilized the same quarterly budget amount for Quarter 3 and Quarter 4 within their internal lost revenue calculations spreadsheet. The error resulted in \$160,417,776 being reported as Lost Revenue in Excess of Total Distributions within the report instead of \$167,748,797, or understating the lost revenue amount by \$7,331,021.

We deemed this to be a significant deficiency in internal controls.

Cause:

The condition found results from the existing internal control review over the accuracy of the underlying data used to prepare the report not being performed at a precision level that detected the error duplicating the budget amounts from Quarter 3 to Quarter 4.

Possible Asserted Effect:

Failure to ensure accuracy of amounts reported as "Lost Revenues in Excess of Total Distributions" may result in HRSA relying on incomplete or inaccurate information associated to the BILH's utilization of PRF funds.

Questioned Costs:

None.

Statistical Sampling:

The sample was not intended to be, and was not, a statistically valid sample.

Identification of Whether the Audit Finding was a Repeat Finding:

This is not a repeat finding.

Recommendation:

We recommend BILH enhance its internal controls over PRF reporting to ensure each of the data elements reported to HRSA are accurate and result in amounts consistent with its underlying records.

Status: Fully Corrected

BETH ISRAEL LAHEY HEALTH, INC. AND AFFILIATES

Schedule of Findings and Questioned Costs Year ended September 30, 2022

Finding 2022-02 – Reporting

Program: COVID-19 Disaster Grants – Public Assistance (Presidentially Declared Disasters)
Federal Agency: U.S. Department of Homeland Security
Assistance Listing Number: 97.036

Criteria:

Title 2 U.S. Code of Federal Regulations Part 200, Uniform Administrative Requirements, Cost Principles, and Audit Requirements for Federal Awards (Uniform Guidance) Subpart F – Audit Requirements §200.510 requires the auditee to prepare a schedule of expenditures of Federal awards for the period covered by the auditee's financial statements which must include the total Federal awards expended as determined in accordance with §200.502. The Uniform Guidance requires the auditor to determine whether the auditee's schedule of expenditures of federal awards is fairly stated, in all material respects, in relation to the audited entity's financial statements as a whole. In addition, the Uniform Guidance places the responsibility for identifying major programs on the auditor and the schedule of expenditures of federal awards serves as the primary basis for the auditor's major program determination. Therefore, appropriate major program determination by the auditor is dependent on the accuracy and completeness of the information in the schedule of expenditures of federal awards.

Condition:

In preparation of the schedule of expenditures for federal awards (SEFA) management did not record the COVID-19 Disaster Grants – Public Assistance (Presidentially Declared Disasters) expenditures on the SEFA which resulted in the SEFA being understated by \$14,249,269.

We deemed this to be a material weakness in internal controls.

Cause:

BILH did not have an adequately designed internal control to identify the error in federal awards expended as such, the amount of the errors could have been greater and not detected by BILH's control environment.

Possible Asserted Effect:

Potential misstatement to the schedule of expenditures of federal awards.

Questioned Costs:

None noted

Statistical Sampling:

The sample was not intended to be, and was not, a statistically valid sample.

Identification of Whether the Audit Finding was a Repeat Finding:

This is not a repeat finding.

Recommendation:

We recommend that BILH implement a more thorough and detailed process for preparing and reviewing the SEFA to identify adjustments that could result in a misstatement of the SEFA. This should include a formalized and detailed review of the SEFA and a reconciliation to the general ledger and financial statements by someone other than the preparer.

Status: Fully Corrected